EPIC Training Manual

PATIENT SERVICES – CHECK IN PROCESS



Patient Services
WOMEN'S HEALTHCARE ASSOCIATES

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Notes:

Notes

Notes:

Forms vs. Visit Types

Note: Always check to see if there are any notes from the Financial Coordinator

Always have the patient verify their Name, DOB, Street Address, Phone # and Insurance

Visit Type	Forms Given
BH RETURN VISIT WHA	Verify that the Behavioral Health Therapy at WHA Form has been signed
BH CONSULT VISIT WHA	Verify that the Behavioral Health Therapy at WHA Form has been signed
BREAST AND PELVIC EXAM WHA (For Est **Medicare Part B** Patients Only)	- Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form - No WBA given - Give patient the Medicare High Risk Assessment form to fill out at their seat *make sure to add a patient label* (give this form every Breast and Pelvic Exam) - Give patient the ABN Medicare Breast Pelvic form – add a patient label and then scan it into the Documents table during the Check-in process
CIRCUMCISION WHA	Treat this as a NP Office Visit – Parent needs to sign consent forms *Check to see if there are any notes from the Financial Coordinator

COLPOSCOPY	-Verify that all consent forms are signed -Verify Permission to Communicate date /info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form
CRYOTHERAPY WHA	None
CYSTOSCOPY WHA	None
ENDOMETRIAL BIOPSY WHA	None
ER FOLLOW UP WHA	Look to see if patient is Est or New If new, follow directions for NP Office Visit If est, follow directions for Est Office Visit
EST OFFICE VISIT	-Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form
EST WELLNESS VISIT WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form

	- Did pt do GYN WBA via MyHealth? If not, give them a paper copy of WBA to complete - Does pt have Medicare as primary insurance (no replacement plans or as a secondary plan)? If so, give patient ABN Wellness form and Medicare High Risk Assessment form (do this every Wellness they come in for)
GYN TRIAGE WHA	Look to see if patient is Est or New If a new, follow directions for NP Office Visit
GYN ULTRASOUND WHA	Have pt sign the GYN Ultrasound Discloser – This is given EVERY time a patient is getting a GYN US. *Needs to have a patient label in the top right-hand corner and then should be interofficed to HIM at BL
INJECTION WHA	None
IUD INSERTION WHA	- Verify there are no notes from the FC team because there might be an additional IUD form(s) to sign or money to collect
IUD REMOVAL WHA	None
IUD REPLACEMENT WHA	-Verify there are no notes from the FC team because there might be an additional IUD form(s) to sign or money to collect
LAB WHA	None

LEEP WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, the patient needs to re-sign the Permission to Communicate form
MAMMOGRAM WHA	- Does pt have Medicare? If so, give patient ABN Mammogram form
NEXPLANON WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form
NP OB TRIAGE WHA	- Give all consent forms
NP OFFICE VISIT WHA	- Give all consent forms -Did pt do WBA via Myhealth? If not give them a paper copy of WBA to complete
NP WELLNESS VISIT WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date /info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the

	Permission to Communicate form -Did pt do WBA via MyHealth? If not, give them a paper copy of WBA to complete -Does pt have Medicare as a **primary** insurance (no replacement plans or as a secondary plan)? If so, give patient ABN Wellness form and Medicare High Risk Assessment form (do this every Wellness they come in for)
NST WHA	None
NURSE VISIT WHA	Look to see if patient is Est or New If new, follow directions for NP Office Visit If est, follow directions for Est Office Visit
OB 3 HR GTT WHA	None
OB CHECK	None – but verify there aren't any FC notes
OB PRIME CONSULT BSUS WHA	Look to see if patient is Est or New If new, follow directions for NP Office Visit If est, follow directions for Est Office Visit
OB TRIAGE WHA	None

OB ULTRASOUND WHA	Look in patient's Appt Desk sidebar to see if there is a note from THIS pregnancy that states "(Date) OB USD" – If **you don't see** that note, give pt the OB Ultrasound Discloser form to sign (see OB USD chapter in the Check-in manual here for full workflow). Put a patient label in the top right-hand corner and then interoffice form to HIM
OB WORKUP WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date /info within Permanent Comments in registration is correct, update if needed. If updates are needed, the patient needs to re-sign the Permission to Communicate form ***(More steps continued on the next page)*** -Did pt do OB WBA via Myhealth? If not, give them a paper copy of the WBA to complete -If patient is getting an Ultrasound alongside this appt, look in patient's Appt Desk sidebar to see if there is a note from THIS pregnancy that states "(Date) OB USD" – If **you don't see** that note, give pt the OB Ultrasound Discloser form to sign (see OB USD chapter in the Check-in manual here for full workflow). Put a patient label in the top right-hand corner and then interoffice form to HIM

PELLETS WHA	Give the Pellets Waiver
PESSARY FITTING WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form
POST-OPERATIVE WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form
PRE OP WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form
POSTPARTUM WHA	*Check to see if there are any notes from the Financial-Coordinator

SIS 70 WHA	Have patient sign the GYN Ultrasound Discloser. This is given EVERY time a patient is getting a GYN US. Put a patient label in the top right-hand corner and then interoffice form to HIM
TRIGGER POINT INJECTION WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form
URODYNAMICS WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form
UROGYNECOLOGY WHA	-Verify that all consent forms are signed -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form - Give the Urogynecology Intake form for new Urogyn patients

VULVAR INTAKE WHA	-Verify that all consent forms are signed *(see more on next page)* -Verify Permission to Communicate date / info within Permanent Comments in registration is correct, update if needed. If updates are needed, patient needs to re-sign the Permission to Communicate form

Check In Process

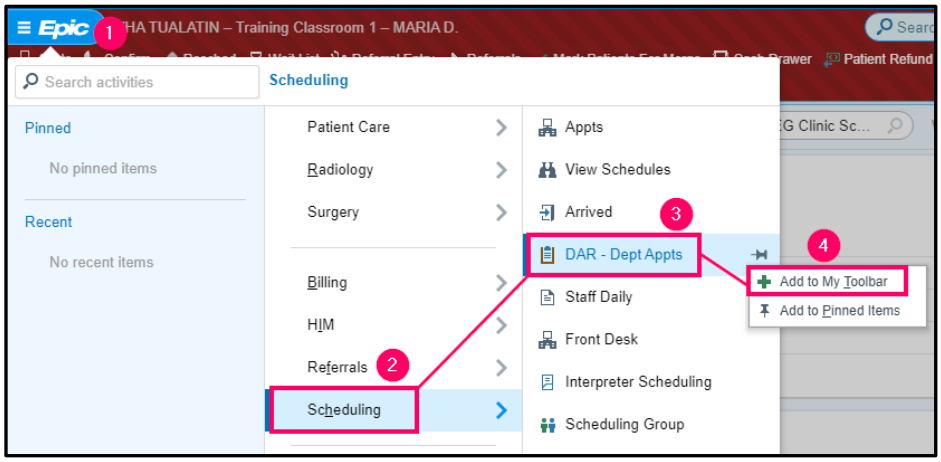
Registration – Editing Established Patient Information

When initially searching for the patient's registration, if the patient does not recognize the DOB/address/or name that you have on your screen, instead of editing it – notify your supervisor either verbally or through a Staff message.

Do not change unless you have the PSS's permission

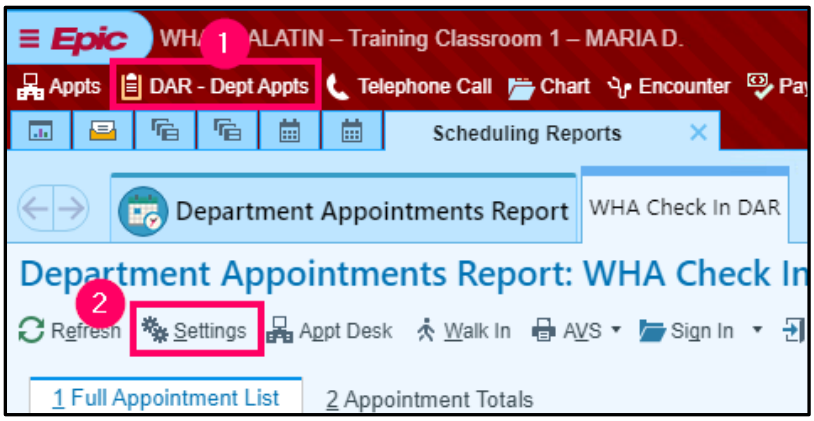
If the patient says they recently had a marital status change or has recently moved, BUT recognizes their old information, then it is ok to proceed with editing the patient's information without a PSS's permission.

Department Appointment Report (DAR)



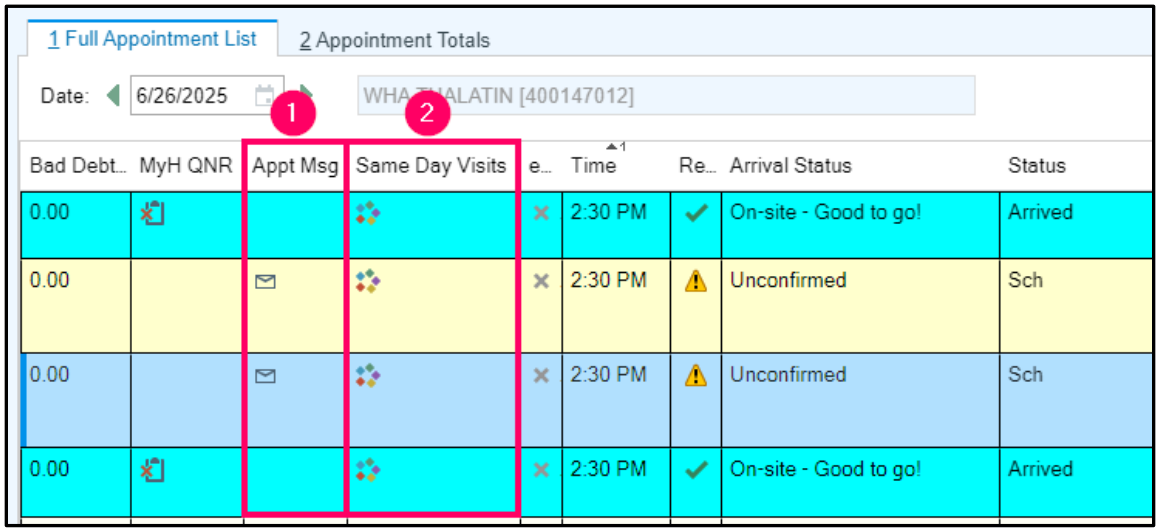
How to add the DAR function to the toolbar:

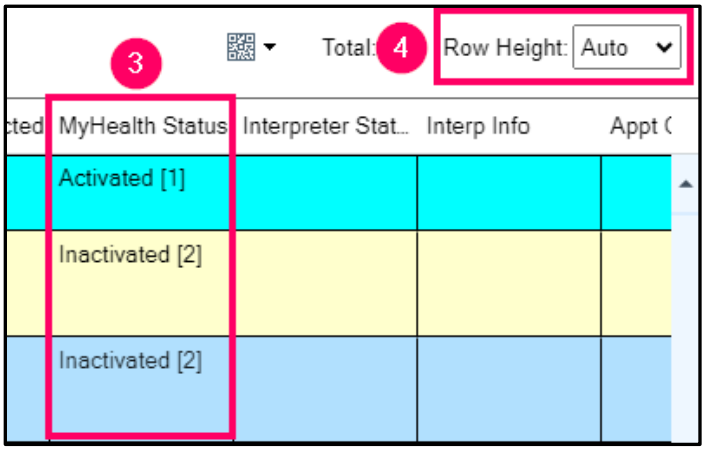
1. Click on the Epic drop down menu
2. Click on Scheduling
3. Right-click on DAR – Dept Appts
4. Choose Add to my toolbar



1. Your Department Appt Report will show up under Scheduling Reports and you can access it by clicking on it from your toolbar

2. DAR options: Click on the **Settings** Icon
   - Change or add departments here
   - Adjust which providers the DAR displays: uncheck the "All" box and add the providers needed below "All"
   - Adjust visit types displayed: uncheck "ALL" and add the particular visit types in the box below "All"
   - Adjust displayed "Appointment status": Uncheck "ALL" and add the particular Appointment Status' below "All"
   - Adjust the time range display to see only a certain time window that moves through the day
   - Click **Save As** to save the report you created and give it a title in the pop-up
   - Set your adjusted DAR as your Default Report
   - Click **Run** to see the DAR



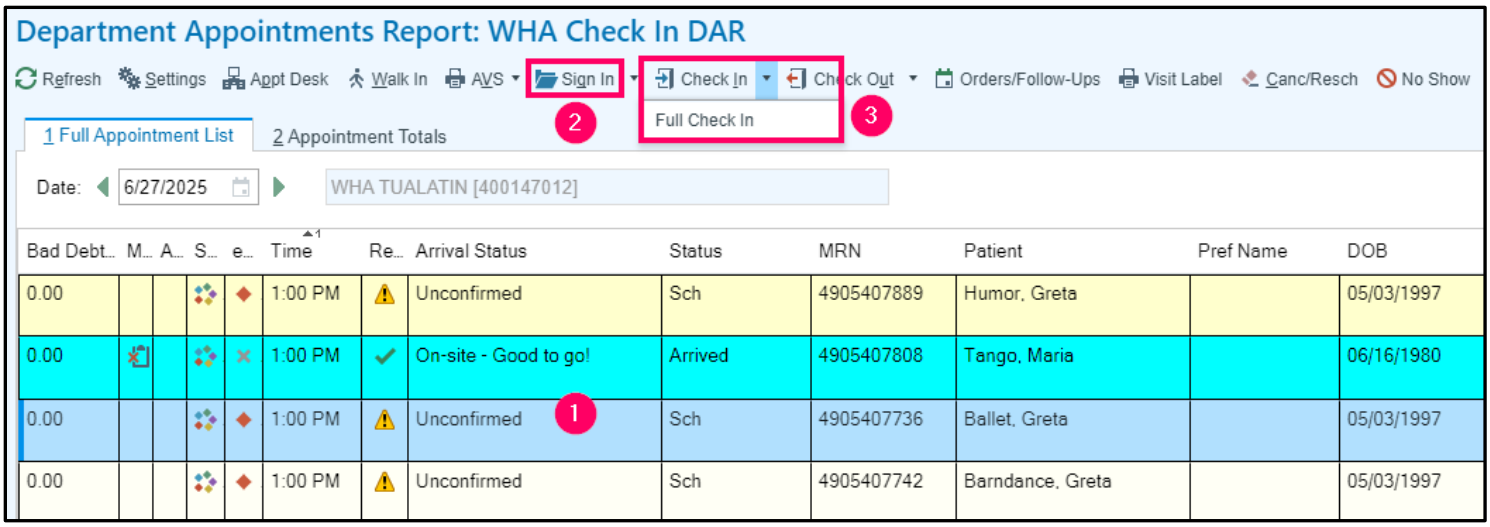


1. This is where Financial Coordinators will leave notes about the patient's account. Hover over the envelope icon to display the message

2. If the patient has more than one appointment in the day, it will display here with:

3. This column displays the patients MyHealth Status

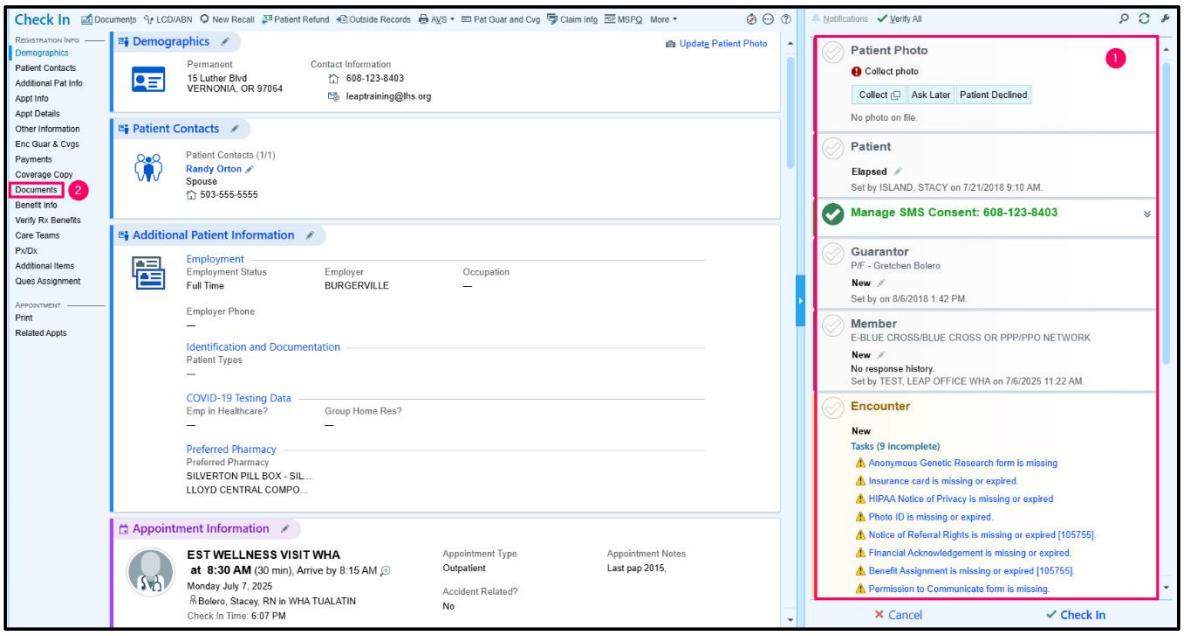
4. You can increase the height of the rows manually by selecting the drop down and choosing the line height options.
   "Auto" will expand any row height that has more than one line of text present while keeping the other rows at their default height

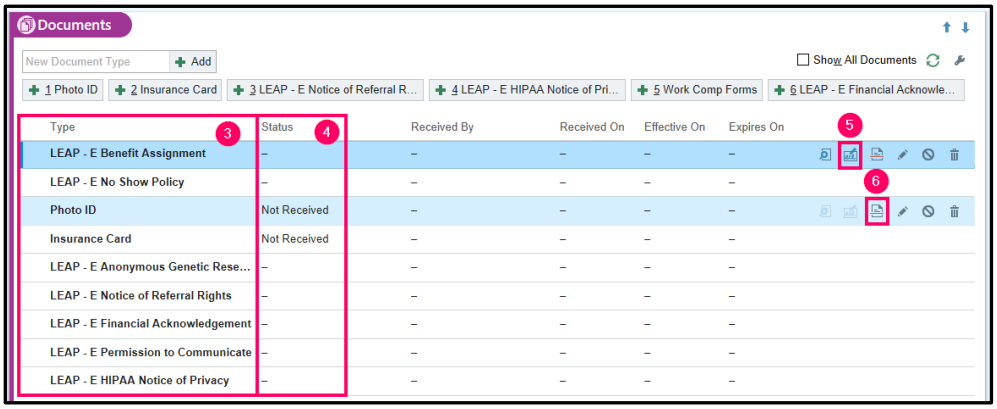


1. Verify that you have the correct patient with Patient Identifiers: **Name/DOB**. Click on the row of the patient you need to check in

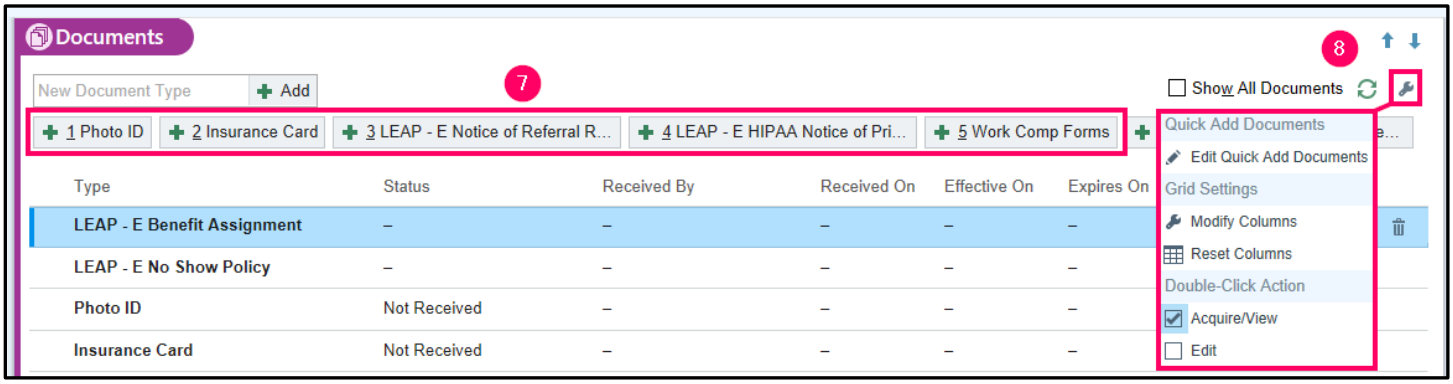
2. Click on **Sign In** then click **Ok**
   *If the patient has more than one appointment scheduled for the day, Sign In all appointments they have scheduled that day. This will change the status of that patient's appointment(s) to "Present"

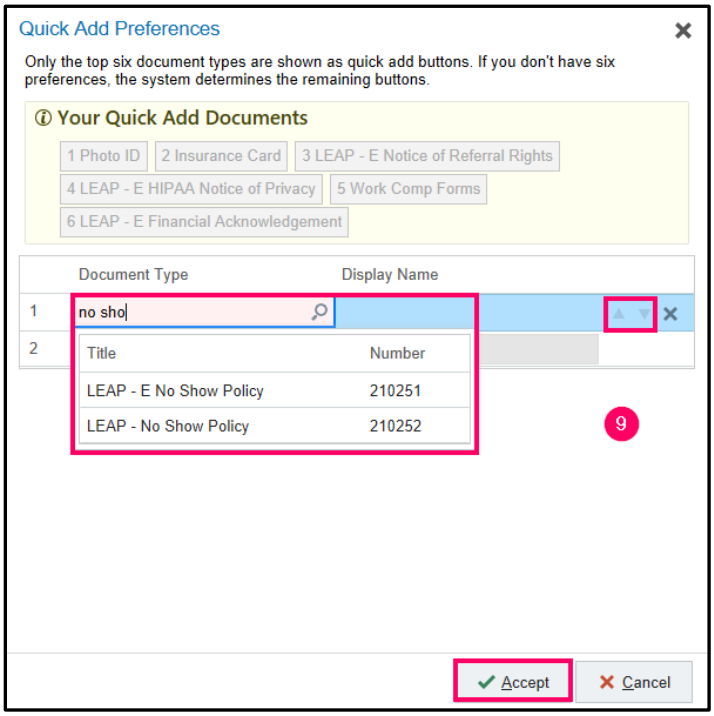
3. Select the drop down next to **Check In** and click **Full Check In**, but only for the appointment the patient is currently checking in for. Other same day visits will remain as Signed In until the patient is ready for them





1. Verify with the patient their address, billing address, phone number, insurance and review any forms the patient needs to sign. The patient photo can also be collected from the Checklist (or you can choose Ask Later or Patient Declined)
2. Click on **Documents** or select the blue hyperlink from the Encounter section in the Checklist to jump to the Documents list
3. The **Type** column lists the Document Types
4. The **Status** column will show if a form has been signed or if documents have been scanned. If the section is blank or states 'Not received" you will need to scan or have the patient E-Sign (*Also indicated in the Checklist under Step #1.)*
5. Select the **E-Sign icon** to have a patient electronically sign the selected form
6. Select the **Document Type** to view the options function. Select the paper icon to scan physical documents (Example: Insurance Card or Photo ID)





1. The Documents table will automatically add **Quick Add Documents** for you to quickly add in more document types

2. To edit these quick buttons, select the **wrench icon** then **Edit Quick Add Documents** from the dropdown menu

3. From the Quick Add Preferences window you can add your Document Types, rearrange their order or remove. Click **Accept** to save

Consent Scripting

Parents/Guardians can sign consent forms for the patient if they are present and willing to sign. If a patient is 15+ years old and presents on their own, then they can sign themselves

Genetic Privacy - Our Genetic Privacy states that you can choose to participate in anonymous or coded genetic research. Would you like to opt in to anonymously donate your health information and biological samples (such as left over blood, urine or tissue samples) for research or would you like to decline and opt out?

Benefit Assignment - The Benefit Assignment gives us permission to bill your insurance on your behalf.

Referral Rights - If we refer you to see a provider outside of our clinic for any reason (such as physical therapy), you have the right to choose whichever provider that would work best for you and your insurance. You will also be financially responsible for those appointments.

Financial Policy - Our Financial Policy states that we will bill your insurance and anything they do not cover, you will be financially responsible for. If you have copays, they will be due at the time of your appointment. We also want to let you know that if you need paperwork filled out that is related to leaves from work (such as FMLA or Short Term Disability), WHA may charge a fee.

HIPAA Privacy - Our HIPAA Privacy Policy states that we will not disclose your personal information without your specific permission. Our HIPAA Privacy Policy is here if you would like a copy.

No Show Policy - Our no show policy states that we request, whenever possible, that you give us at least 24 hours' notice when you cannot make an appointment so that we can offer that time to another patient in need of care. If you cancel your visit with less than 24 hours' notice or no show on more than 3 occasions, it may affect your ability to be seen in our office.

Permission to Communicate – (Don't Forget to Expire Doc 1 Year Out)
This form gives us permission to communicate with anybody in your personal life such as a spouse, parent, sibling or child, if they were to call in on your behalf. Is there anyone that you would like to list for us to be able to communicate with?
If so, would you like for them to have access to all information, medical information only, financial information only or scheduling only?

Detailed Voicemail: When we call you, is it ok to leave detailed voicemails? If so, which phone number is best and would you like us to leave All, Medical, Financial, Scheduling information or a combination of these options?

GYN Ultrasound Disclosure - This is our Pelvic Ultrasound Disclosure. The top portion explains that there may be different methods used to perform your ultrasound, which can include abdominal, transvaginal and possibly 3D imaging. Though ultrasound is a valuable tool to find abnormalities, not all abnormalities may be identified.

The Financial Disclosure section states that the different methods of ultrasound used will appear on your statement as separate charges. Ultrasound costs may be covered by your insurance, however, the charges may be applied to your deductible.

OB Ultrasound Disclosure - This is our OB Ultrasound Disclosure. The top portion states we have the technology for Ultrasound, but we can't always see everything on the Ultrasound. Videotaping or photography, by camera or cell phone, is not allowed during the exam. Printed pictures will be provided to you if you would like. In addition, a flash drive of your ultrasound is available for purchase upon request.

The Financial Disclosure states that the ultrasound costs may be covered by your insurance, however, the charges may be applied to your deductible.

The second page of the OB USD is for the patient to keep and gives some information on ultrasounds. Also, don't forget to add "(date) OB USD" to a Patient Message in the patient's Appt Desk after they sign the OB version

ABN General Scripting

"The next form is a Medicare based form that we need to review before your visit. It has information on the top about what possibly could be done today and the associated fee. Next, we need you to choose and initial an option below. **Option 1** is stating you want to be seen today and you do want us to bill Medicare, **Option 2** is you do want to be seen today but you don't want us to bill Medicare, and **Option 3** is you do not want to be seen today and you don't want us to bill Medicare"

Well Being Assessment Scripting

[*After scheduling*]

Shortly you will receive an email saying that you received a secure message from Women's Healthcare Associates. You will then want to log on to your MyHealth Patient Portal using the login and temporary password that I gave you, and you will find the email from us in your inbox.

Now, it will have your appointment dates, times, and locations at the top, but it will also have a link down below that takes you to a form called the Well Being Assessment.

This form is really helpful to us as it gives us an idea of what resources we can provide and how we can support you as a patient. It takes about 5-10 ten minutes to fill out and ideally is filled out before your first visit with us so we can get you any needed resources as soon as possible. If you have any issues with the form, you are more than welcome to give us a call.

If you are unable to fill this form out online, we ask that you please arrive 20-30 minutes prior to your scheduled appointment time.

[*If a patient does not have an email*]

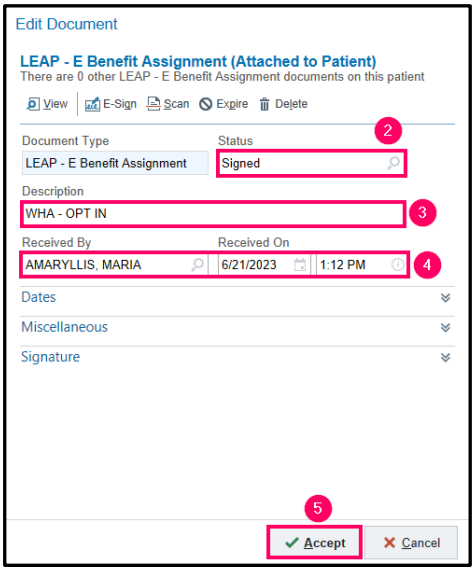
This form is really helpful to us as it gives us an idea of what resources we can provide and how we can support you as a patient. It takes about 5-10 ten minutes to fill out and ideally is filled out before your first visit with us so we can get you any needed resources as soon as possible. I am going to mail you a printed copy today to your address on file and if you have any questions with the form, you are more than welcome to give us a call. Pease make sure to bring it to your first visit!

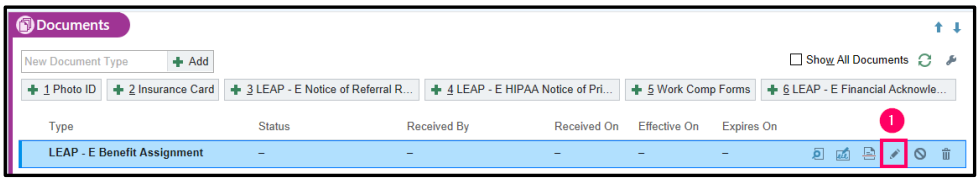
[*If a patient does not speak English*]

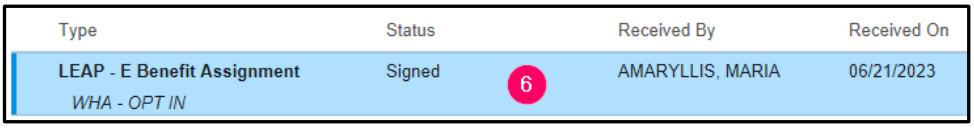
Prior to your first visit we will have you fill out a form called the Well Being Assessment. This form is really helpful to us as it gives us an idea of what resources we can provide and how we can support you as a patient. It takes about 10-15 ten minutes to fill out and ideally it is filled out before your first visit with us so we can get you any needed resources as soon as possible.

We will arrange for your interpreter to arrive with you 30 minutes prior to your first appointment so they can assist you with your forms.

Document Descriptions







1. Once a form is signed or a document is scanned, select the document and click the pencil icon to edit the entry

2. Verify that the Status is set as "Signed" for E-Sig Forms or "Received" for scanned documents

3. Enter one of the following Descriptions:

Photo ID = LEAP WHA *
* Driver's License = State initials (Ex. OR, HI, IL, etc.) – DL
* Passport = PASSPORT
* Student ID = STUDENT ID
* Military ID = Military ID

Insurance Card = LEAP WHA – Full name of the insurance. If secondary, indicate in the description

Consent Forms:
* Anonymous Genetic Research = **WHA**
* Benefit Assignment = **WHA**
* Notice of Referral Rights = **WHA**
* Financial Acknowledgment = **WHA**
* HIPAA Notice of Privacy = **WHA**
*No Show Policy = **WHA**
*Permission to Communicate = **WHA**

4. Verify that your name and today's date/time is entered under the **Received By** section

5. Click **Accept**

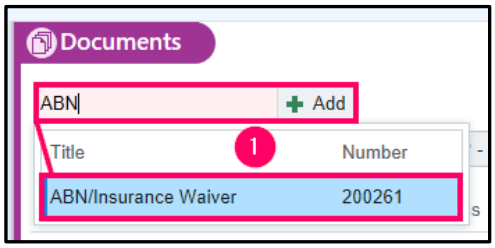
6. The Document Description, Status and Received By and Date will be displayed on the Document Table

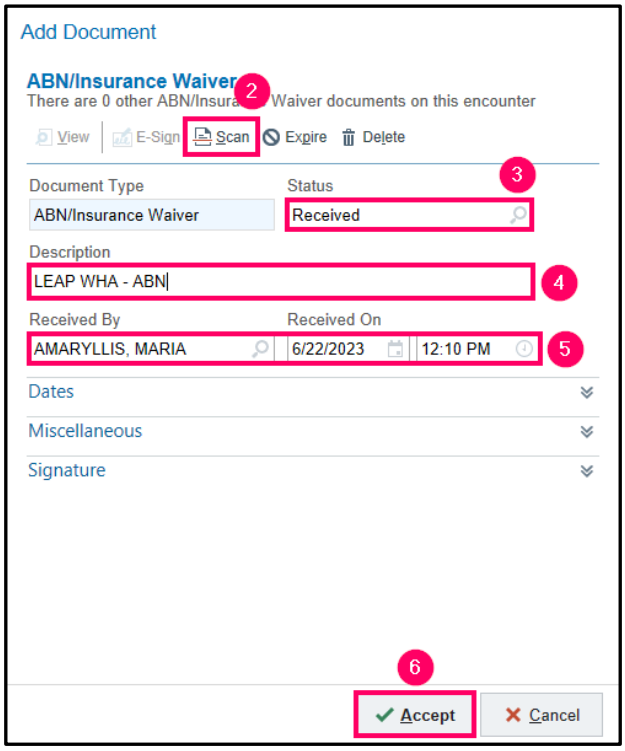
Signing and Scanning Medicare ABN Forms

Link to the ABN Form Scripting (under the Consent Scripting chapter)

FYI: ABNs are encounter specific – you need to go to the visit and right click to choose the **Reg Appt Contact** option to be able to scan the ABN to the Documents table or scan them during the Check-in process with the patient

For ABN Wellness & Mammo forms: Patient needs to fill out/sign the form before checking them in completely for their appt. When they are done, scan under the Documents tab (scanning must be done during Check-In or within the Reg Appt Contact option)







1. To add an ABN, search "ABN" in the +Add section on the Documents Table
2. Select Scan to scan the paper form then go back in to Edit the document
3. Verify that the Status is set to "Received"
4. Add a Description: "**LEAP WHA – ABN**"
5. Verify that your name, the scan date/time are entered correctly
6. Click Accept
7. The Description and Received information will display on the Documents Table

For ABN Lab forms: The patient will complete this form if it is needed, with the Lab Assistant. The Lab Assistant will then place it faced down in a designated spot by the Check-In desk for the PS team to scan into the patient's Registration Document section. VERIFY there is a correct patient label placed in the top right-hand corner prior to scanning (instructions on how to print the correct patient label **here**)

Printing copies of the ABN from Sharepoint: *Clinical and Patient Services* > *Patient Services forms/letters/consents* > *Check In Forms*. Choose the appropriate ABN form for the visit type

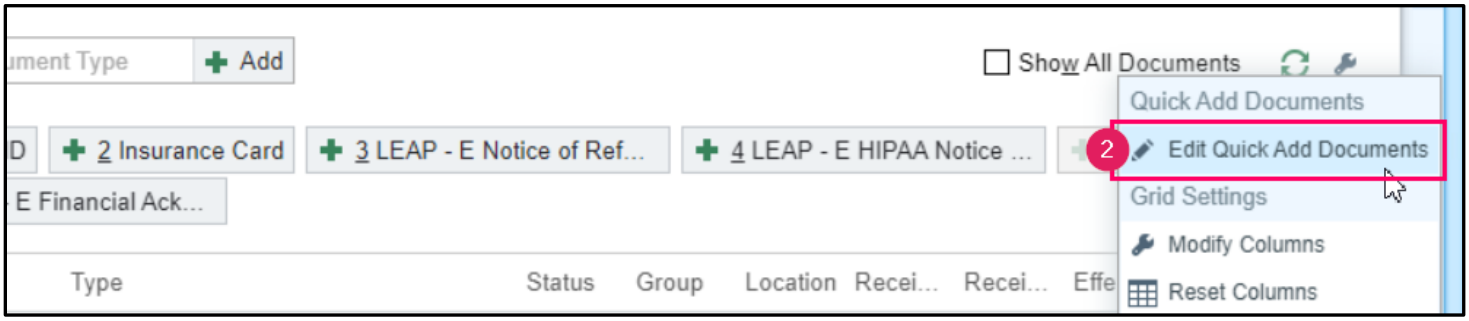
If you would like to learn more about the Medicare High Risk Form and its workflow, click this link below:

Medicare High Risk Questionnaire Workflow Link

Adding Custom Speed Buttons for the Documents Table

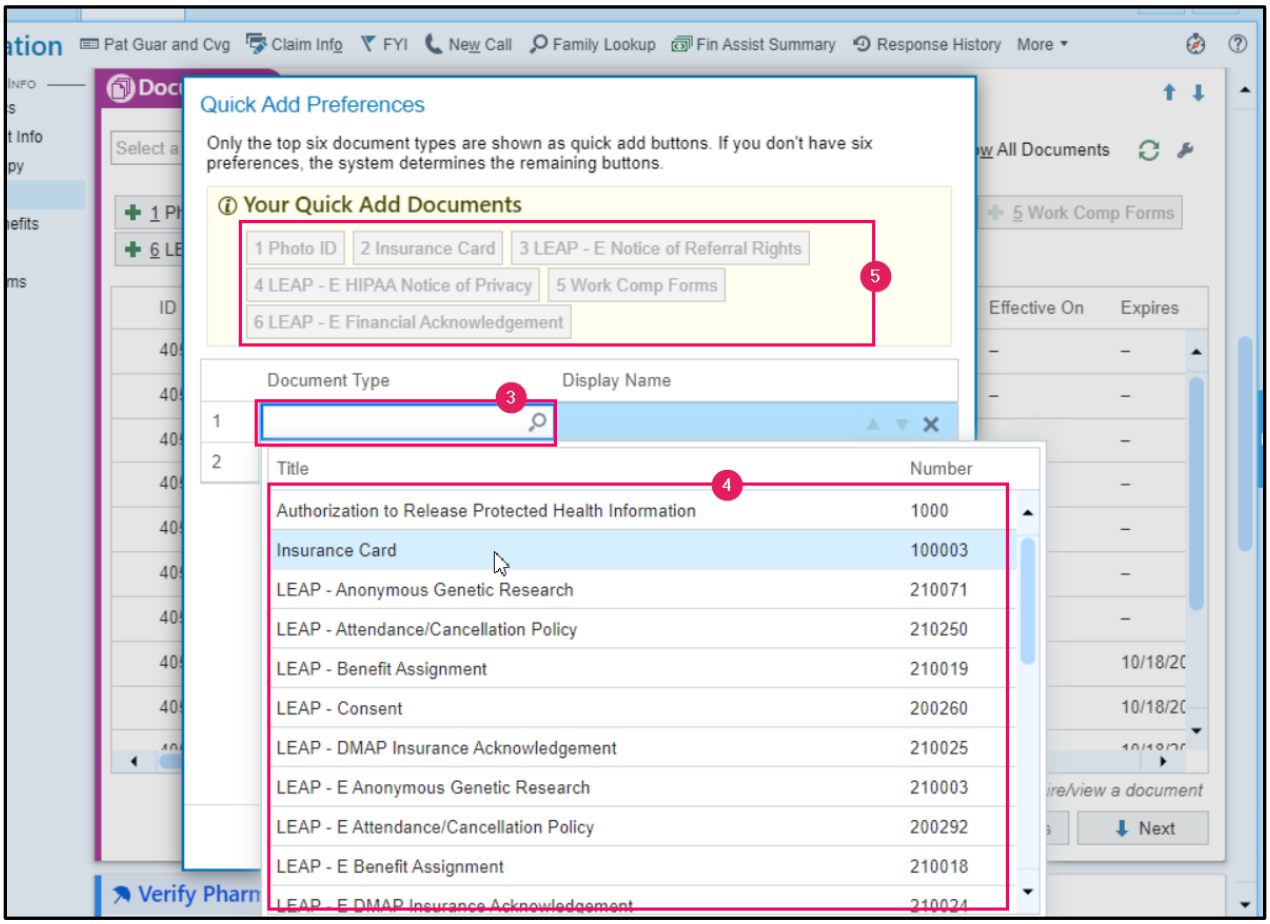
Document type speed buttons can be customized above the document table. You can choose which 6 document types that you would like to show for quick use. Common selections are ABN Form, Insurance Card, Photo ID, and Permission to Communicate (the non-e-sign version)

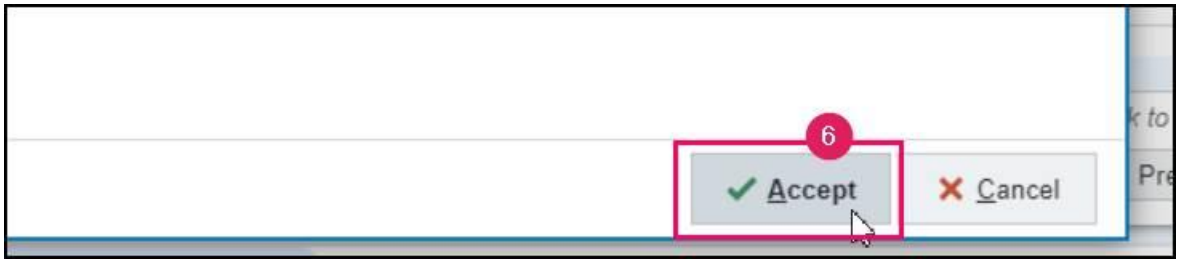




1. Click on the wrench in the top right corner of the documents section within Registration

2. Click on Edit Quick Add Documents within the drop-down menu





3. Click on the magnifying glass to search the list
4. Choose a document you would like to save as a speed button
5. You can choose up to 6 total
6. Click Accept to save your custom preferences

Signing and Processing OB / GYN USD Forms

The Ultrasound Discloser form should be given:

GYN Ultrasounds = Every Ultrasound

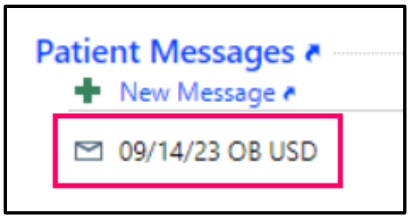
OB Ultrasounds = Only their 1st Ultrasound per pregnancy. Although, we should be checking every OB Ultrasound to see if the appropriate notes are in the Appt Desk Sidebar/Storyboard because **if you do not see the notes** it may mean another Receptionist forgot this form and the patient will need to sign it with you.

FYI: The OB USD has 2 pages and the patient should take the second page with them

Once the patient has signed the Ultrasound Discloser form:

GYN Ultrasounds = Print a patient label from today's visit and place it in the top right-hand corner of the form. Send form to HIM team at BL in appropriate inner office mail folder.

OB Ultrasounds = Create a new Patient Message on the patient's Appt Desk and type **"Today's date – OB USD".** This will alert future Receptionist that the patient has signed this form for this pregnancy. Then print a patient label from today's visit and place it in the top right hand corner of the form. Send form to HIM team at BL in appropriate inner office mail folder.



Link to OB USD form on Sharepoint to print more copies

Link to GYN USD form on Sharepoint to print more copies

Permission to Communicate

FYI's about this document:

- Patients need to sign the Permission to Communicate form every year (usually at their Wellness visits, OB Workups, or when they turn 18) and / or if the patient has legally changed their name

- Due to this change, the Permanent Comments will need to be updated (date & names) each time the form is re-signed

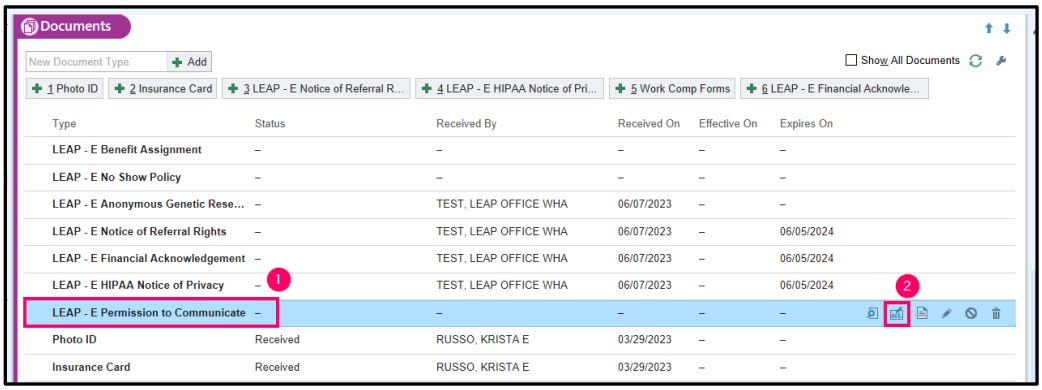
- If the patient checking in can read, write and understand the form they need to be the one that signs the form (at any age)

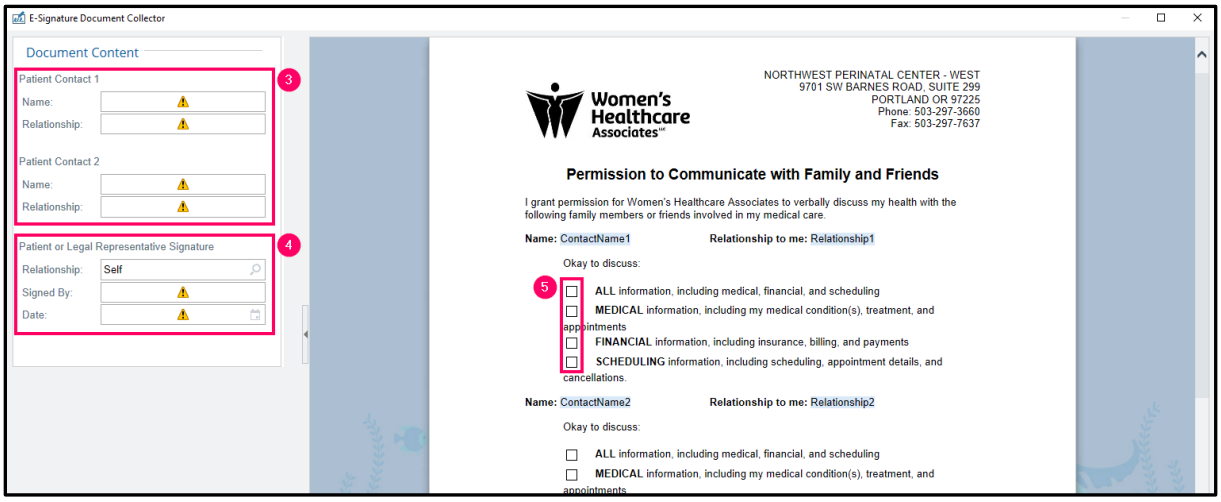
- If the patient cannot read, write or understand the form then their guardian can sign for them but that guardian needs to put themselves as a contact on the form

- For HIPAA reasons, never delete a signed form - always expire

- When collecting a new Permission to Communicate form, the Check-In desk will add a expiration date of one year out from the patient signing it *(ex: if the form is signed on 5/2/24 the expiration date will need to be listed as 5/2/25)*

- A designated PS team member will be going through the next day's DAR to expire any Permission to Communicate forms that have been signed over a year ago





1. In the **Documents** tab on the Navigator Pane in Registration, click on the **LEAP – E Permission to Communicate** line

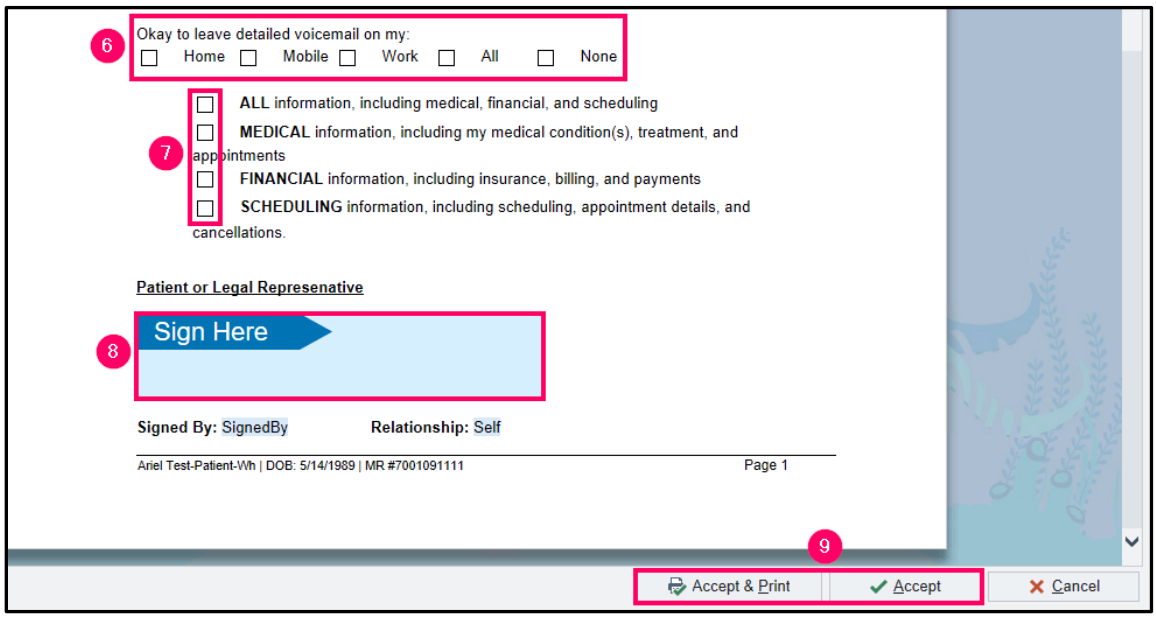
2. Click on the **E-Sign button**

3. Enter the Name and Relationship of the person(s) the patient is giving permission for us to communicate with. You can add 2 contacts per document
   *If the **patient declines** then leave these sections blank

3. Patients of all ages who are able to read, sign their name, and understand the form should complete the form themselves. They can include their family, parent(s) or any legal guardians they wish us to communicate with on the form as contacts

   If the parent or guardian does sign the form, they need to list themselves as a HIPAA ok to talk to contact on the form

4. **Checkmark** a box to indicate what type of information can be released to each contact. Multiple boxed can be checked if not selecting ALL. *(Example: Financial only)*



6. Ask the patient if it is ok to leave detailed voicemails on one or more preferred numbers. Checkmark the appropriate box(es).
   Note: Make sure to verify the patient's contact numbers are correct in Registration

7. Verify which information is ok to be left on a detailed voicemail.
   Multiple boxes can be checked if not selecting ALL *(Example: Financial & Scheduling only)*

8. Click **Sign Here**. The patient will then use the signature pad to electronically sign the form
   Note: The patient will need to sign this form even if they decline release of information and/or detailed voicemail

9. Click **Accept** or **Accept & Print** if the patient would like a copy

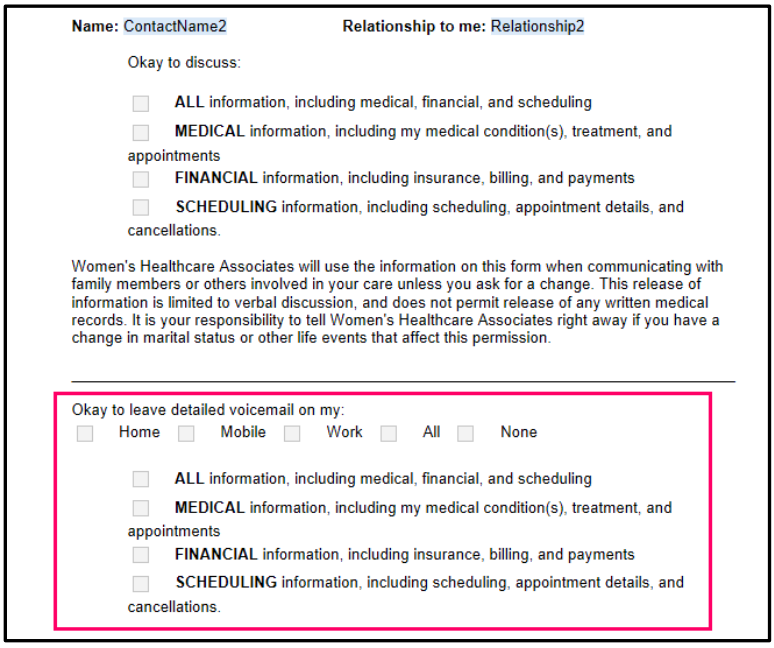
Detailed Voicemails

Background: There has been a significant increase in HIPAA breaches by our staff leaving detailed voicemails for patients when **they have not** given us permission to do so. This updated workflow will help gather patient consent for leaving Detailed Voicemails.

Check-In Workflow:

At Check-in, the Patient Services staff will ask each patient (using the Permission to Communicate form) and document in the Permanent Comments if they are ok with WHA leaving Detailed Voicemails on their phone (the patient will also have an option to decline)

Note: **All patients** will need to sign the Permission to Communicate form **every 365 days** to ensure this information is correct and up to date. If the patient wants to update either the Permission to Communicate contacts or their option of leaving voicemails, they need to re-sign the Permission to Communicate form in person



Name: ContactName2 **Relationship to me:** Relationship2

Okay to discuss:

☐ **ALL** information, including medical, financial, and scheduling

☐ **MEDICAL** information, including my medical condition(s), treatment, and appointments

☐ **FINANCIAL** information, including insurance, billing, and payments

☐ **SCHEDULING** information, including scheduling, appointment details, and cancellations.

Women's Healthcare Associates will use the information on this form when communicating with family members or others involved in your care unless you ask for a change. This release of information is limited to verbal discussion, and does not permit release of any written medical records. It is your responsibility to tell Women's Healthcare Associates right away if you have a change in marital status or other life events that affect this permission.

Okay to leave detailed voicemail on my:

☐ Home ☐ Mobile ☐ Work ☐ All ☐ None

☐ **ALL** information, including medical, financial, and scheduling

☐ **MEDICAL** information, including my medical condition(s), treatment, and appointments

☐ **FINANCIAL** information, including insurance, billing, and payments

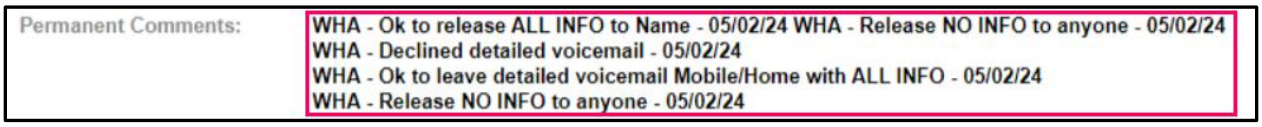
☐ **SCHEDULING** information, including scheduling, appointment details, and cancellations.

When the patient chooses either Home/Mobile/Work/All, the person at Check-In will confirm the phone number(s) at that time

- Staff will document the patient's answer under the Permanent Comments:
  - "WHA - Ok to leave detailed voicemail (type of phone) with All/Financial/Medical/Scheduling info - Date"

  or

  - "WHA - Declined Detailed Voicemail - Date"



- WHA will be accepting and documenting verbal permission to leave Detailed Voicemail messages until the patient is able to come into the office and sign the Permission to Communicate form

Standard Detailed vs. Non-Detailed Voicemail Scripting:

Generic non-detailed voicemail scripting:

"Hello, this is your doctor's office reaching out regarding ***, please give us a call back at xxx-xxx-xxxx"

Detailed voicemail scripting:

"Hello, this is Women's Healthcare Associates calling to reach out to you regarding ***. We would like to ***. Please give us a call back at xxx-xxx-xxxx so that we can ***."

Example of a Detailed voicemail would be:

"Hello, this is Women's Healthcare Associates calling to reach out to you regarding your missed appointment with our office. We would like to reschedule this visit with your provider so we can see you for your medical needs. Please give us a call back at (503) 855-1600 to get rescheduled"

Phone Call/Scheduling Workflow:

- Patient calls to schedule visit and after scheduling, staff reviews Detailed Voicemail question alongside confirming their Care Team provider

- Staff documents whether the patient gave us verbal permission or not to leave Detailed Voicemails under the Permanent Comments in Registration:
  - "WHA - Ok to leave detailed voicemail (type of phone) with All/Financial/Medical/Scheduling info - Date" or
  - "WHA - Declined detailed voicemail - Date"

- **Note:** This verbal permission will be confirmed the next time the patient comes into the office by signing or re-signing the Permission To Communicate form

Telephone Encounter (for call backs) Workflow:

- Patient calls requesting a call back and staff creates a Telephone Encounter and reviews with the patient if it is ok to leave a Detailed Voicemail for this and future return calls

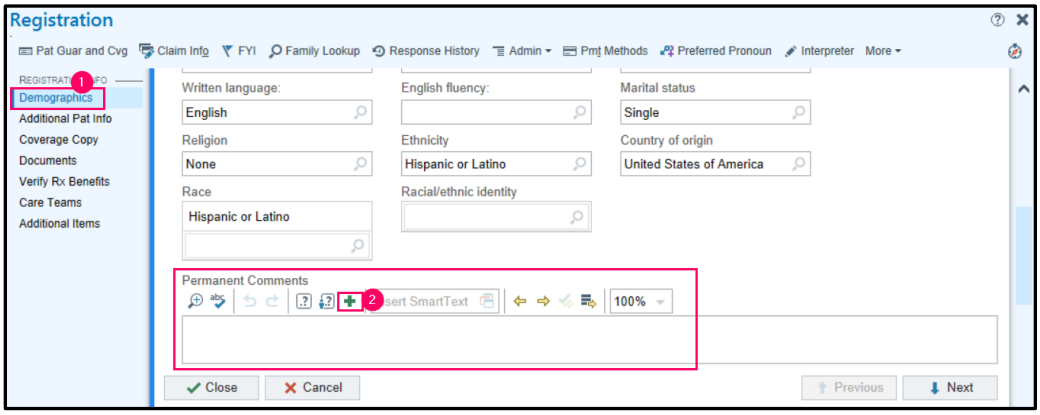
- Staff documents under the Permanent Comments in Registration:
  - "WHA - Ok to leave detailed voicemail (type of phone) with All/Financial/Medical/Scheduling info - Date" or
  - "WHA - Declined detailed voicemail - Date"

- **Note:** This verbal permission will be confirmed the next time the patient comes into the office by signing or re-signing the Permission To Communicate form

Permanent Comments

The Permanent Comments box is used by all LEAP clinics to document HIPAA notes / voicemail notes (such as Permission to Communicate names, needed one time only verbal patient ok's, or if it is ok to leave a voicemail or not). This box lives under the Demographics section within the Registration.

Adding and Using Smartphrases within Permanent Comments for Standardization:

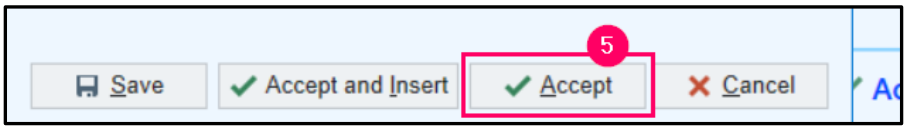


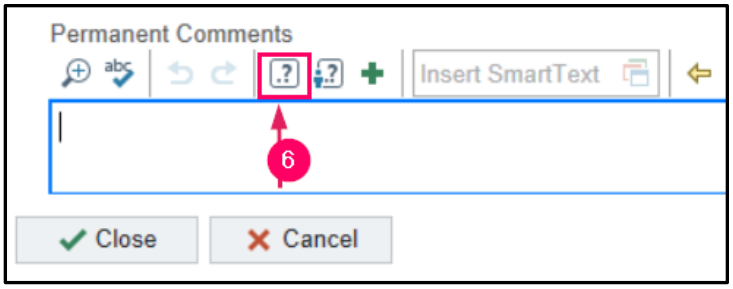


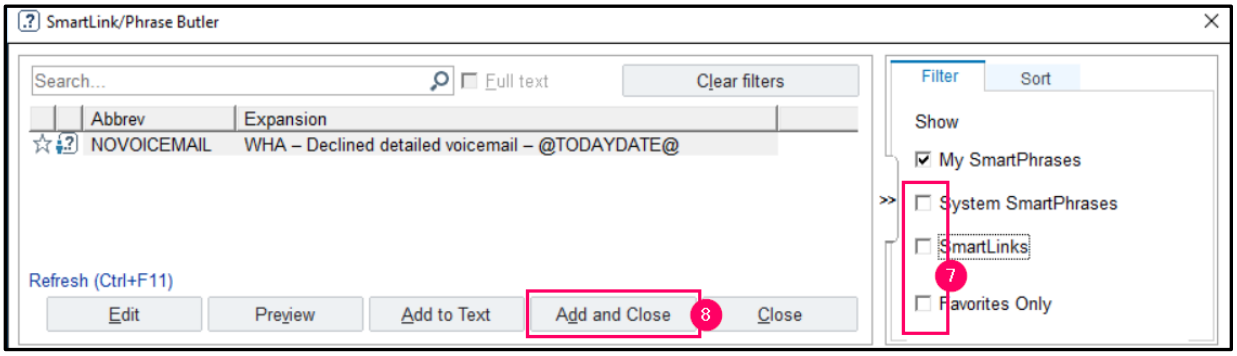
1. Locate the Permanent Comments box within the patient's Registration
2. Click on the Green + icon to add a Smartphrase as a custom favorite
3. For each creation, add one Smartphrase at a time. The 4 Smartphrases we have are:

   .wharoi .whanoroi .whavm .whanovm

4. Name your custom Smartphrase (no spaces or dashes)







5. Click **Accept**
6. Click on this first **question mark** icon (utilize this icon when needing to pull in the Smartphrases you have added)
7. Uncheck these 3 boxes
8. Click **Add and Close**

View on the Appt Desk of possible Permission to Communicate options:

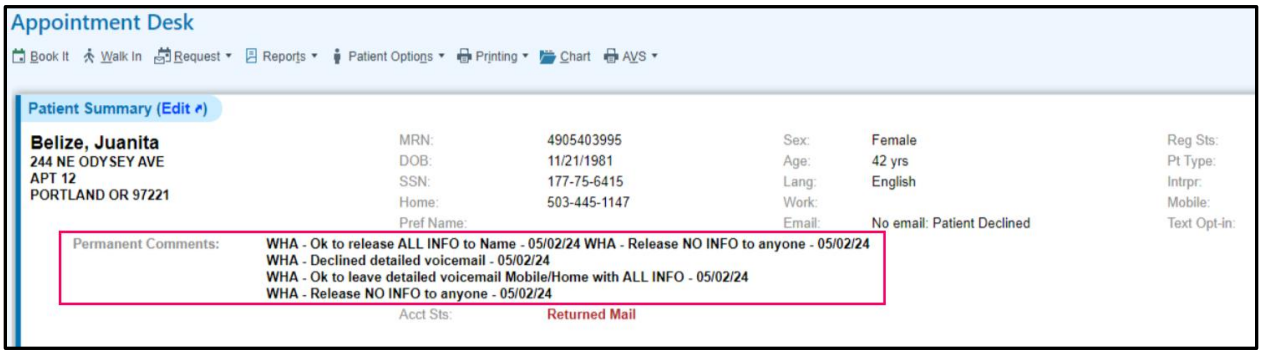
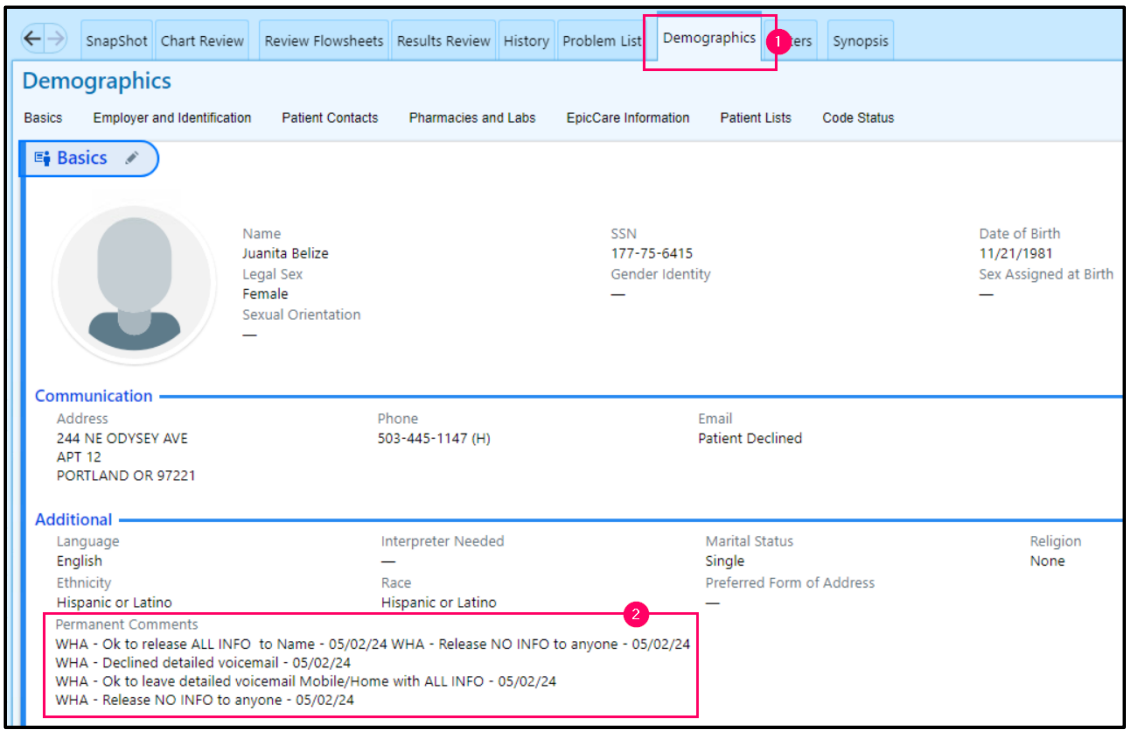
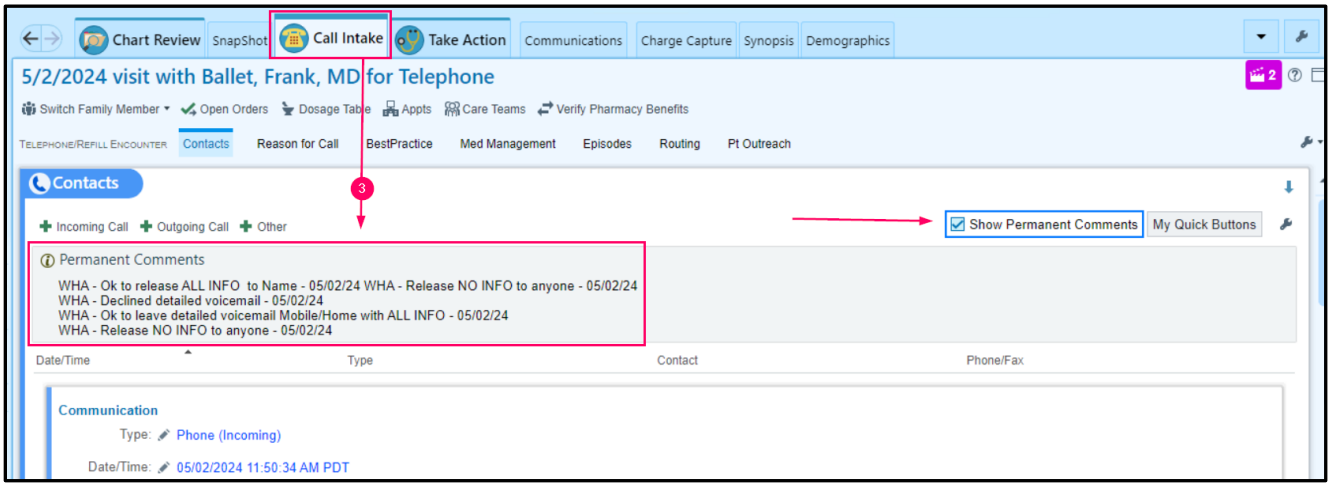


Chart View for Permanent Comments





1. From the patients **Chart**, click on **Demographics**

2. Permanent Comments will display here

3. Permission To Communicate will also display in Telephone Encounters under Contacts – but note, you have to verify this "Show Permanent Comments" box is checked

Collecting Copays

Why is it important to collect a copay at the time of service?

Not collecting copays at the time a service is provided can be costly to WHA. For example, it can cost WHA almost as much as what a patient's copay is to send out a statement. Sometimes we have to mail out several statements before a patient will pay. In this case WHA is actually losing money trying to collect that copay.

Note: WHA is a specialist office so we will always collect the Specialist Copay amount

Always ask to collect a copay when a pt. is being seen for:

- New patient office visit
- Established patient office visits
- Repeat pap
- Triage visit (OB or GYN)
- Genetic consult (NWP only)
- MFM Consult (NWP only)
- Mod risk consult (NWP only)
- ER follow ups

Don't ask for a copay when a pt. is being seen for (but don't refuse if the pt. offers):

- Procedures
- Ultrasounds (OB or GYN)
- Labs/phlebotomy

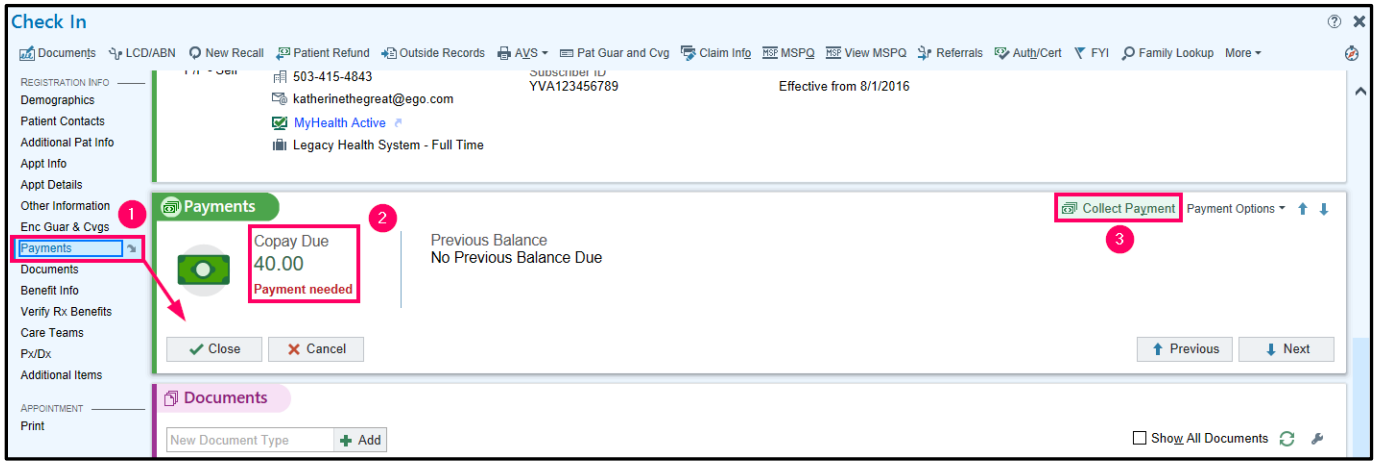
Don't ask for a copay when a pt. is being seen for:

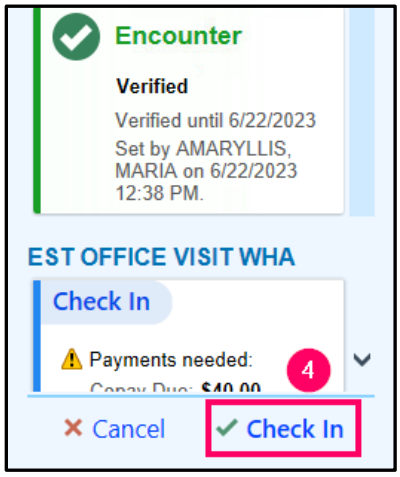
- New patient Wellness visit
- Established patient Wellness visit
- Mammograms
- Routine OB visits

Most services that fall in the yellow category (procedures, ultrasounds, etc.) will cost the patient more than just their copay but every plan is different, and if the patient is prepared to pay their copay at the time of service we should collect it. Any additional balance will be billed to the patient after their insurance processes the claim.

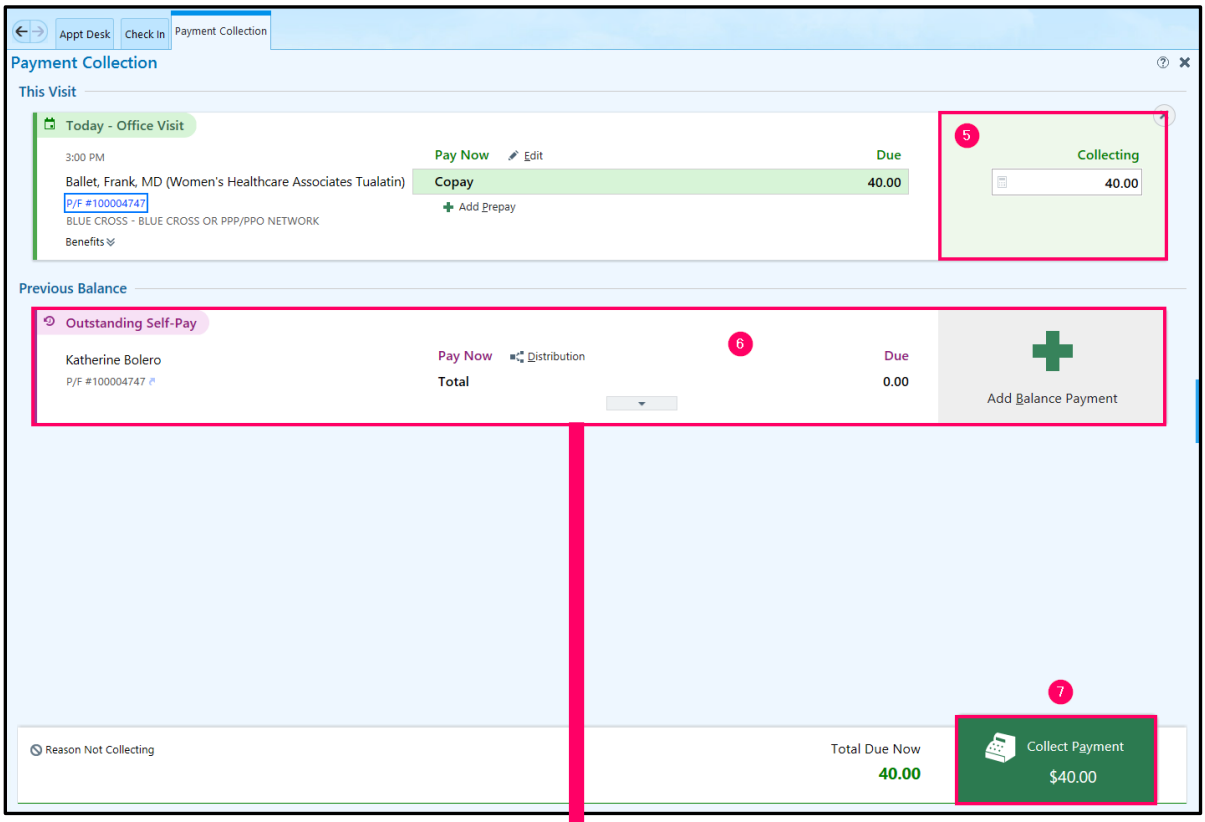
Please don't ever tell patients that there won't be a bill for the service they are receiving. Although it is rare, there are times when an insurance does not pay in full for a mammogram or wellness visit. It is better to tell the patient something like:

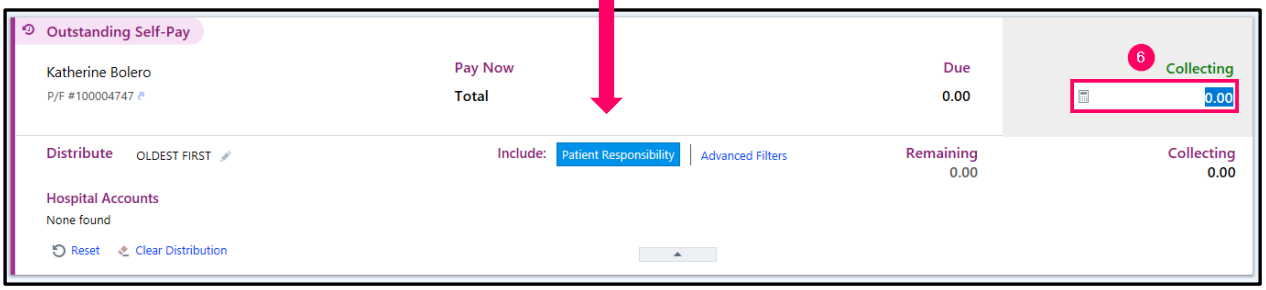
"Let's wait on collecting your copay today. Most of the time insurances pay preventative services in full, but if there is a balance that your insurance does not pay, we will send you a statement."





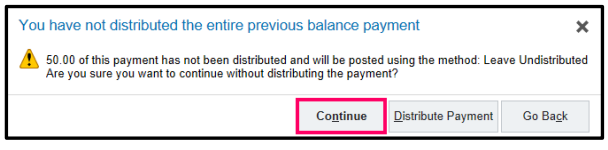
1. To collect payments, go to the payments tab within Registration
2. If a copay is due, it will be listed as **Payment needed**
3. Select **Collect Payment** to go to the Payment Collection screen
4. If you do not select the payments tab before checking the patient in, any due payments will cause the Payment Collection screen to pop up after selecting Check In



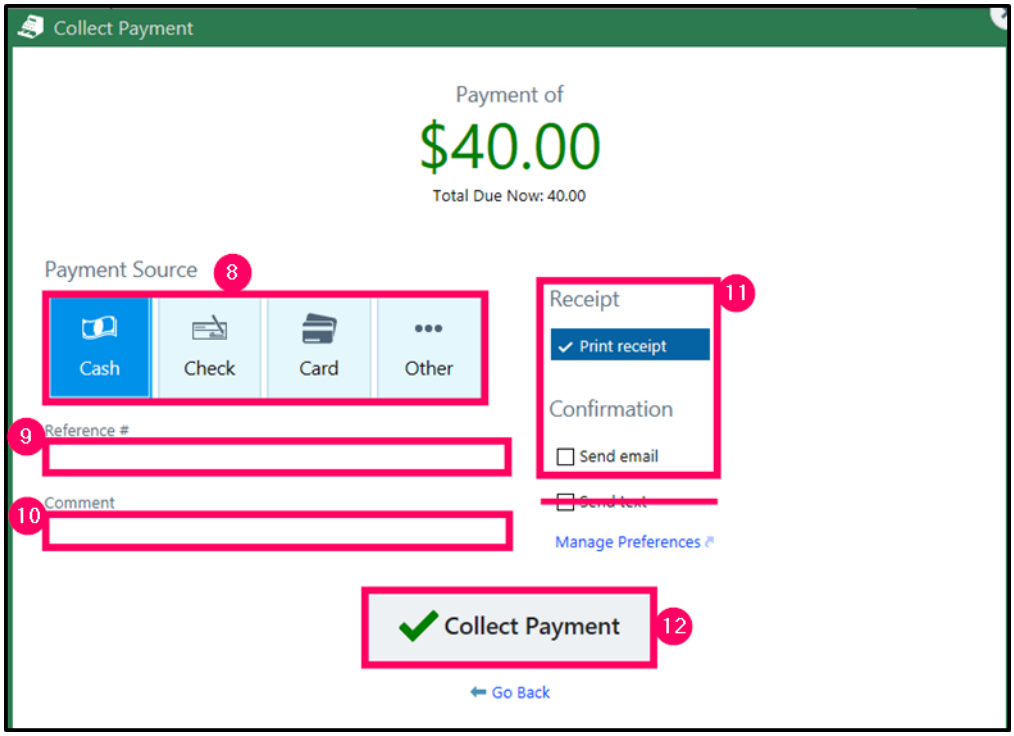


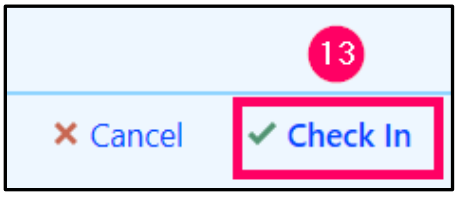
5. The **Copay** amount will be automatically listed under the Collecting field
   See Page 25 for steps if the patient can only pay a partial amount or is unable to pay

6. If the patient has an **Outstanding Self-Pay** balance (balance on the account), it will be listed here. Click **Collect Balance Payment** and then enter the amount. **Continue** through the Distribute Payment window



7. Click **Collect Payment**





8. Click on the form of payment that the patient is paying with

9. **Reference** - If the patient pays by:
   *Check – Enter the check number here
   *Credit Card – Enter the last four digits of the card number here

10. Add any **Comments** if needed. *Example: "Patient could only pay $20 of their $50 copay."*

11. Always give the patient a receipt when taking a payment. You can send the receipt via email or print a copy out

12. Click **Collect Payment.** *Note: Card will prompt the credit card machine to function after selecting Collect Payment*

13. Click **Check In**

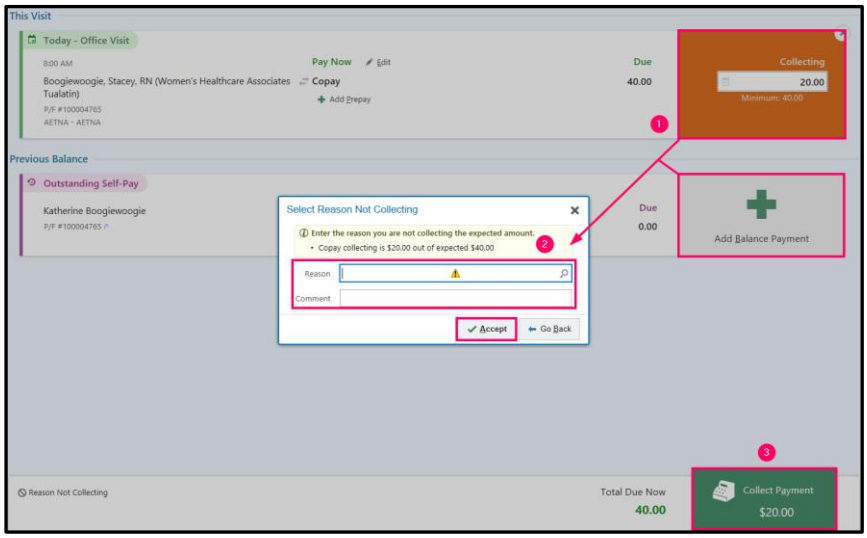
Bad Debt...	M...	A...	S...	e...	Time	Re...	Arrival Status	Status	MRN	Patient
0.00				✗	8:00 AM	✓	On-site - Good to go!	Arrived	4905407768	Ballet, Katherine
0.00				✗	8:00 AM	✓	On-site - Good to go!	Arrived	4905407774	Barndance, Katherine
0.00				✗	8:00 AM	✓	On-site - Good to go!	Arrived	4905407705	Bolero, Katherine
0.00				✗	8:00 AM	✓	On-site - Good to go!	Arrived	4905407779	Boogaloo, Katherine

14

14. The patient is now fully checked in. Their status has changed to "**Arrived**" which indicates to the back office that they are ready to be called back for their visit

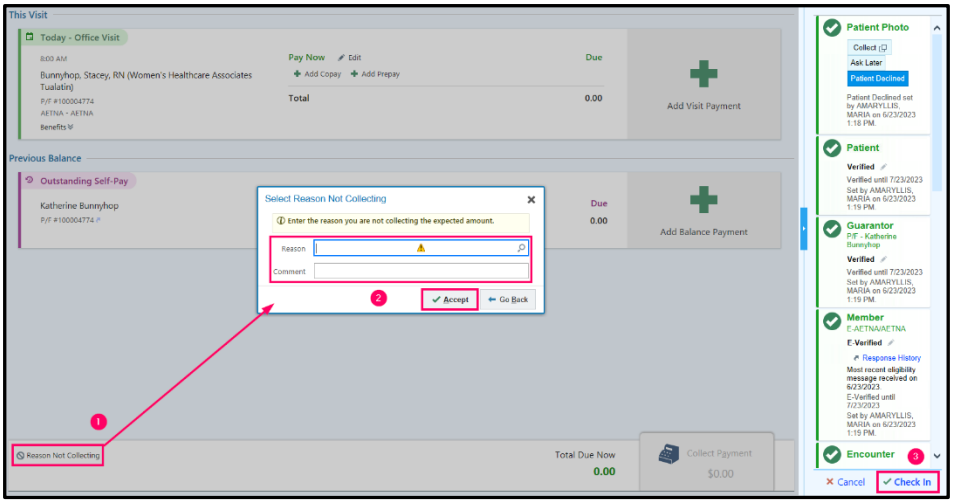
Not Collecting or Partial Payment

If patient is unable to pay or has partial payment towards their copay or their due now balance you must enter a reason why



Partial Payment:

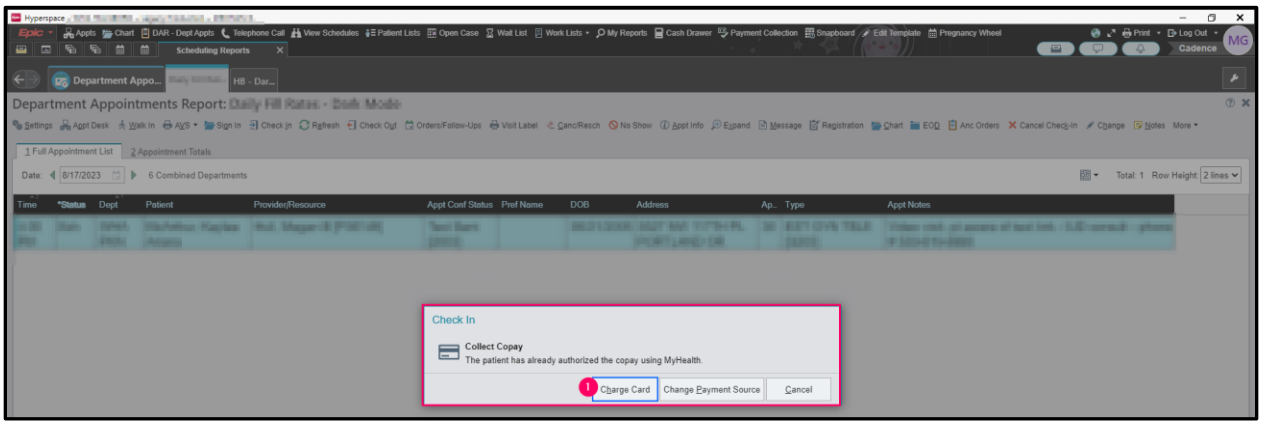
1. Enter the amount the patient is able to pay
2. The Reason Not Collecting window will open. Choose a **Reason**, add a **Comment** then click **Accept**
3. Click **Collect Payment**

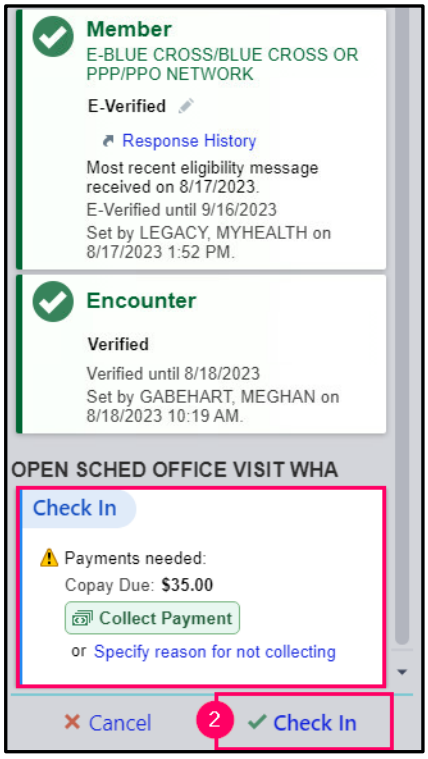


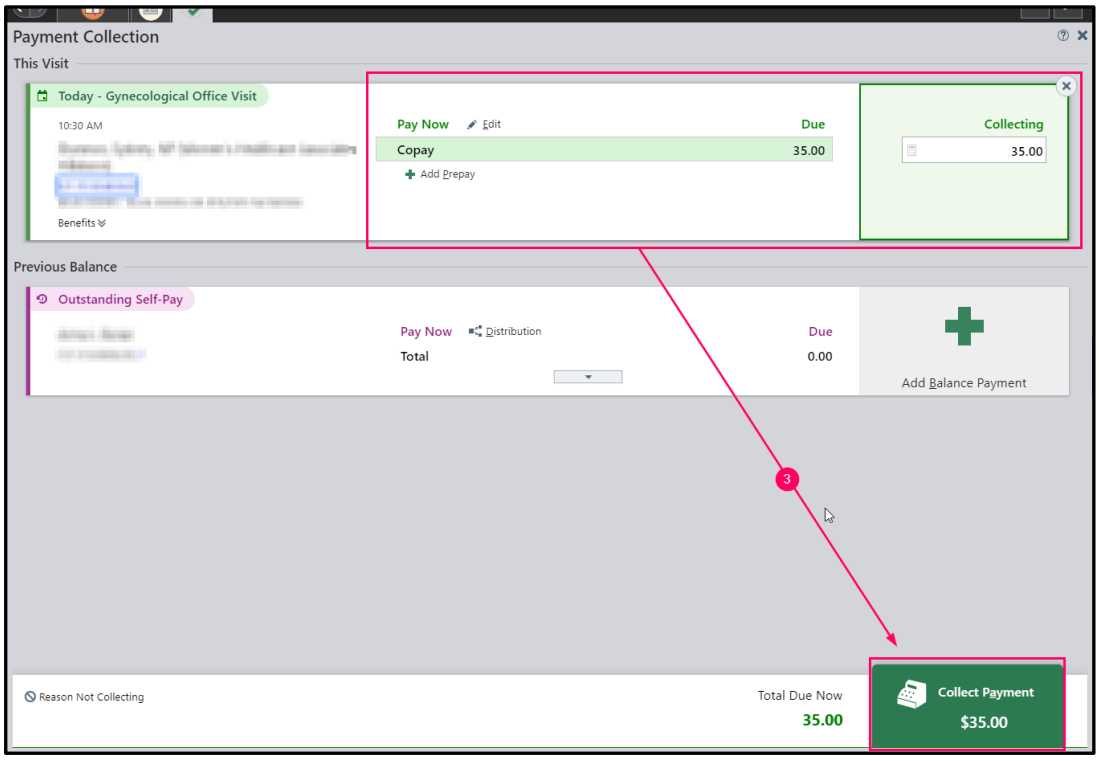
Not Collecting:

1. Click **Reason Not Collecting**
2. The Reason For Not Collecting window will open. Choose a **Reason**, add a **Comment** then click **Accept**
3. Click **Accept**

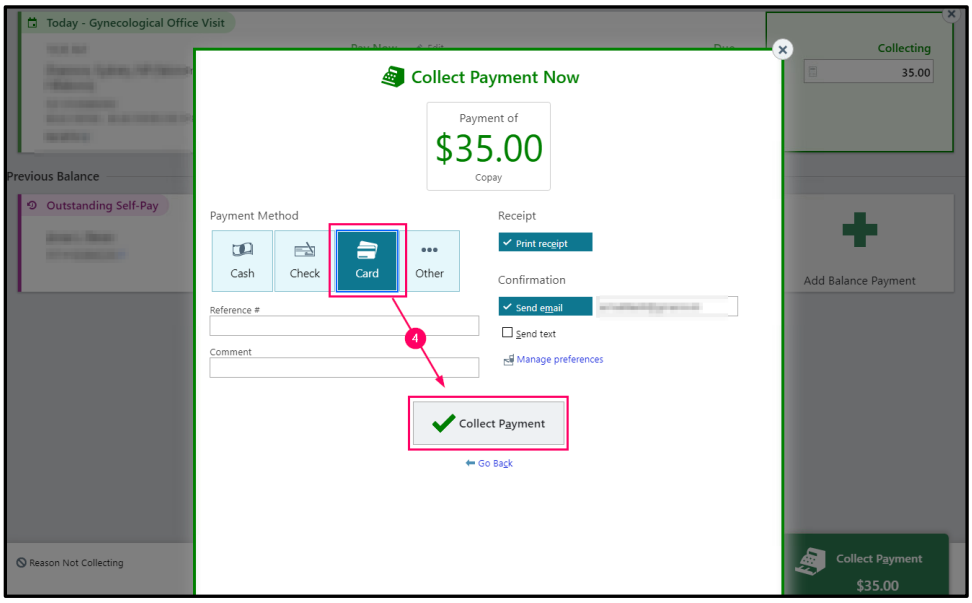
Credit Card – Swipe & Save

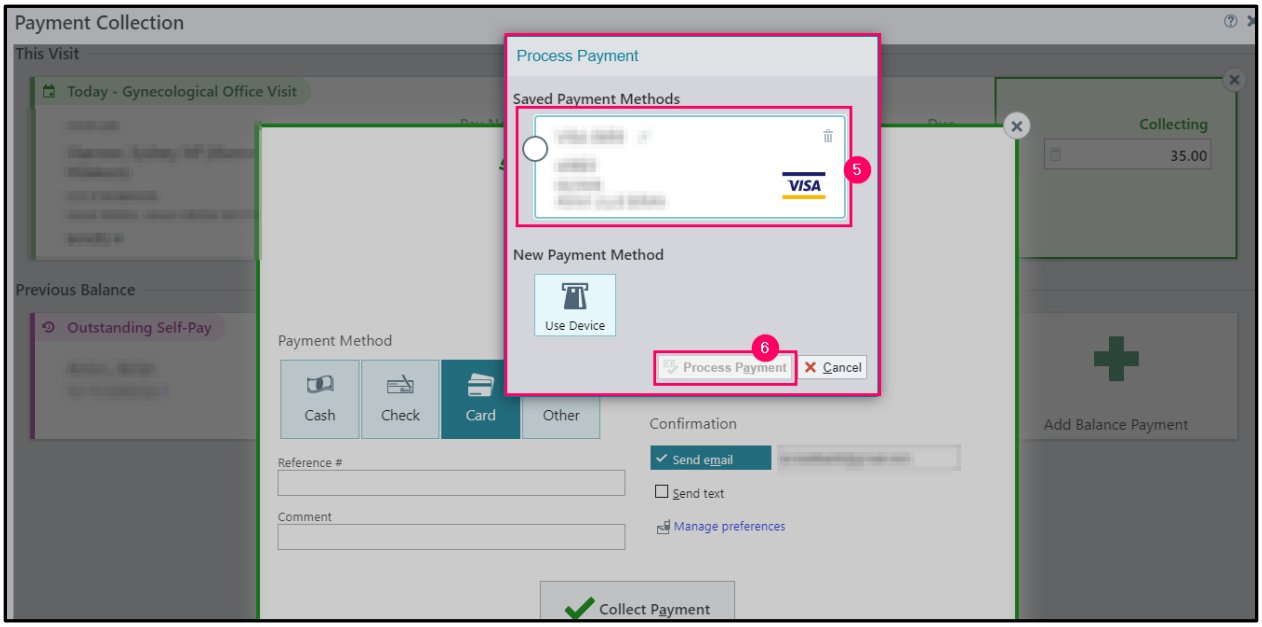






1. When checking in a patient, if they have a card on file you will see a Collect Copay pop up. Always ask the patient if we should charge the card on file or select another payment source. If you cannot verify with the patient which option to choose (example: checking in a telehealth visit) or if the patient cannot remember which card is on file, choose Change Payment Source to go through the normal check in process.
2. After verify required information at check in, click **Check-in** under the checklist
3. The copay will be visible under today's visit. Click **Collect Payment** at the bottom



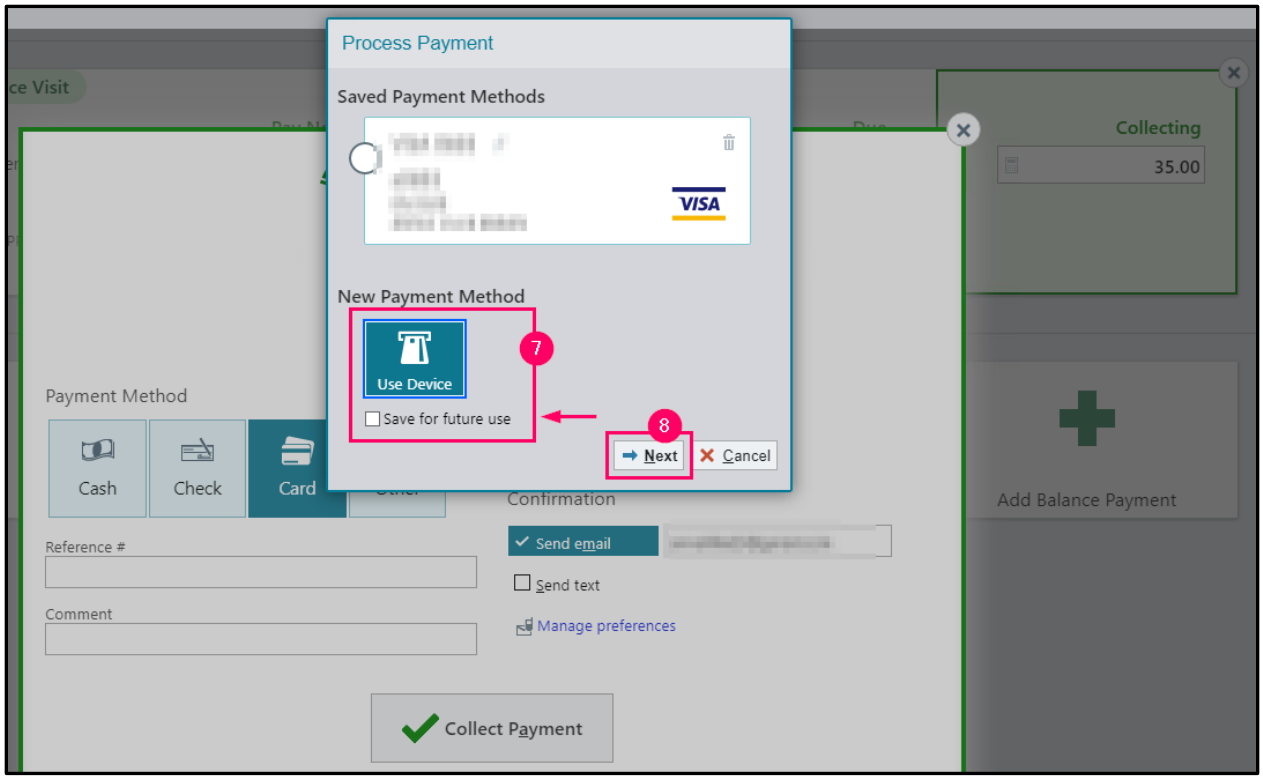


4. If the patient would like to use a card on file or a new card, click Card and then Collect Payment
5. A Process Payment window will appear. **To use the saved card,** click the bubble next to the card listing

Only save cards if they are personally the patient's or their spouse – do not save parent's cards

6. Choose Process Payment

To use a **New Payment Method**, see steps on next page

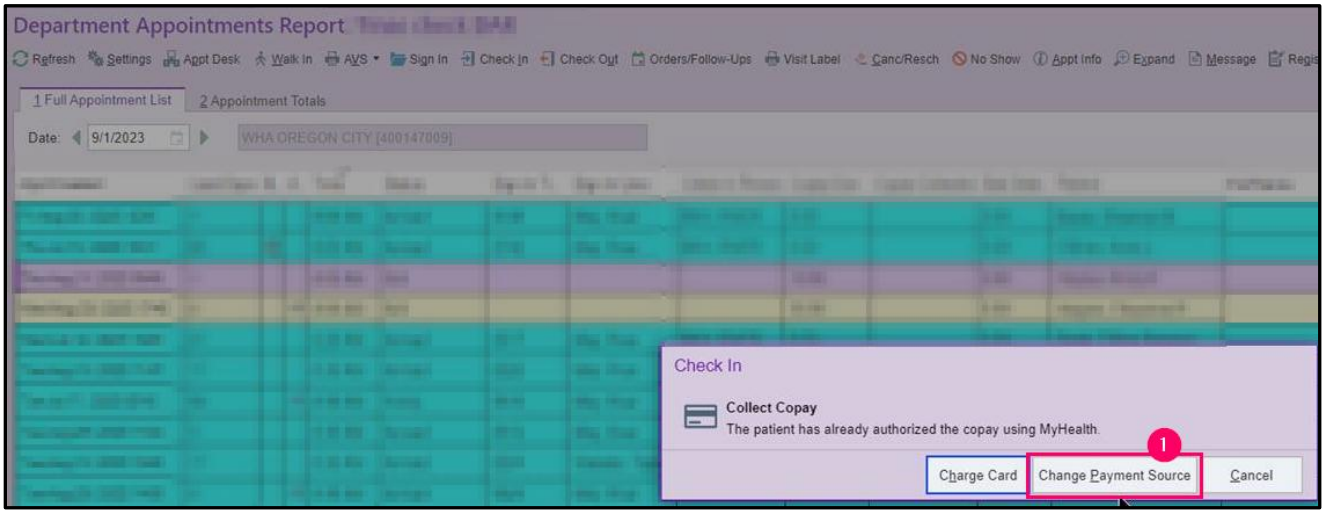


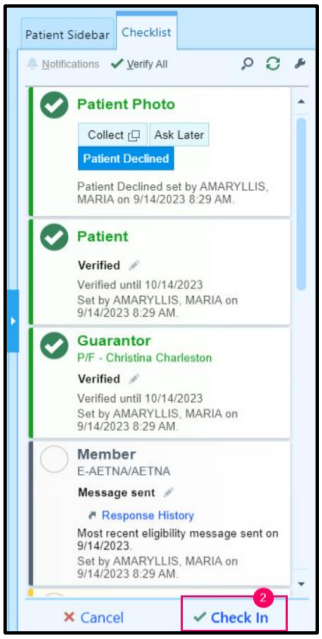
7. **If the patient does not want to use the card on file**, choose Use Device. Always ask before choosing save for future use

8. Choose **Next** to move onto the last steps of the check-in process

Note: Remove any cards the patient does not want on file any longer. Keep this list clean – try not to have 6 listed for a single patient

Checking in Telehealth Visits that have a Copay

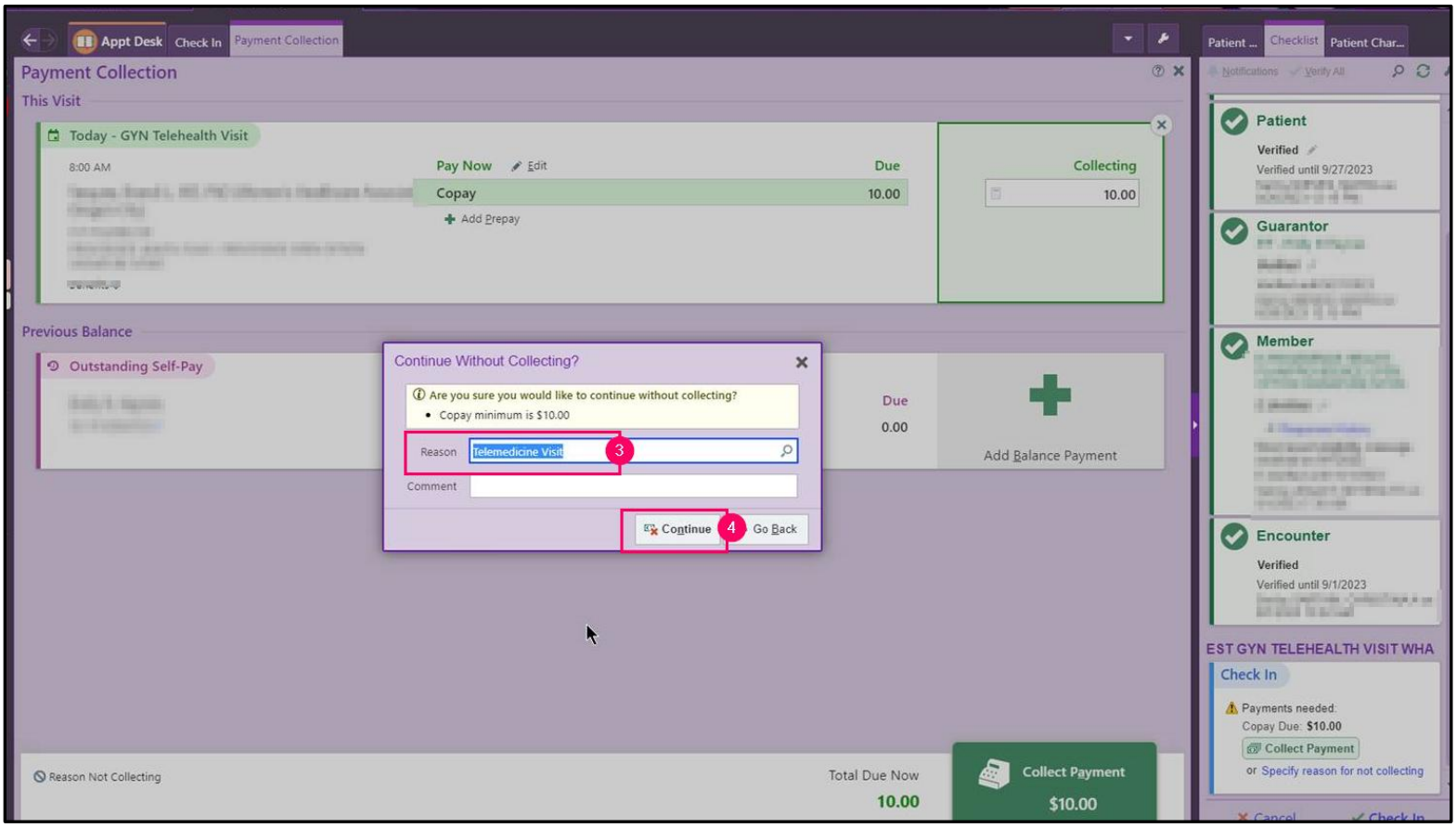




1. When tasked with checking in Telehealth visits on the DAR, there are a few windows to be aware of. First, check in the specific Telehealth visit and then choose Change Payment Source if you see this pop up appear.

Note: we **do not** want to charge the patient's card for Telehealth visit copays until they have spoken to a Financial team member regarding their visit

2. Click Check-in in the bottom right hand corner of the Checklist

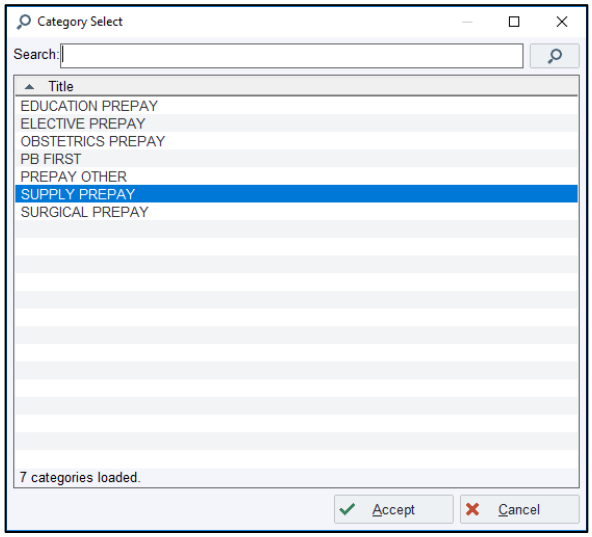
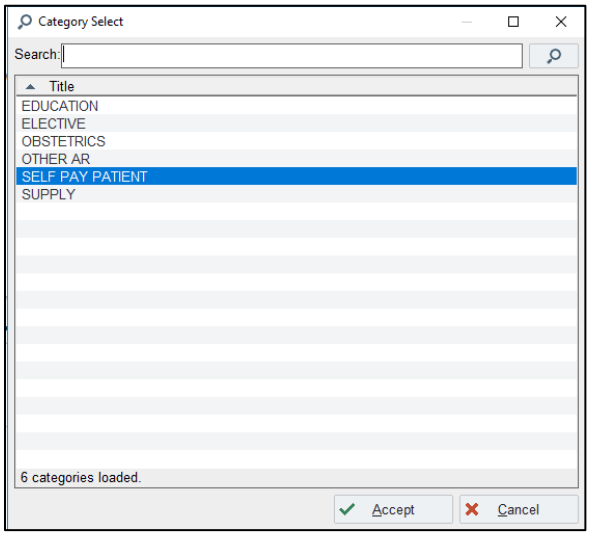


3. To continue without collecting a copay you must type in a Reason. For this scenario, type “Telehealth visit”

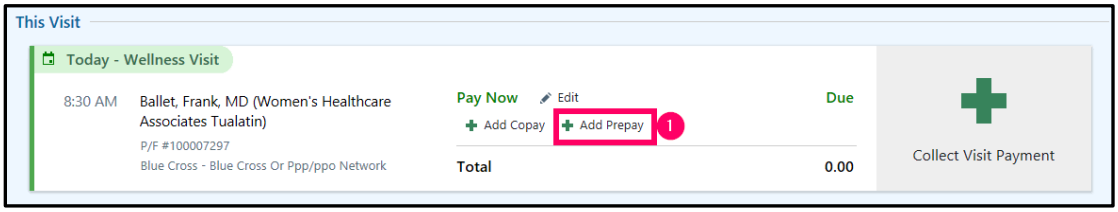
4. Click continue to finish up the check in process

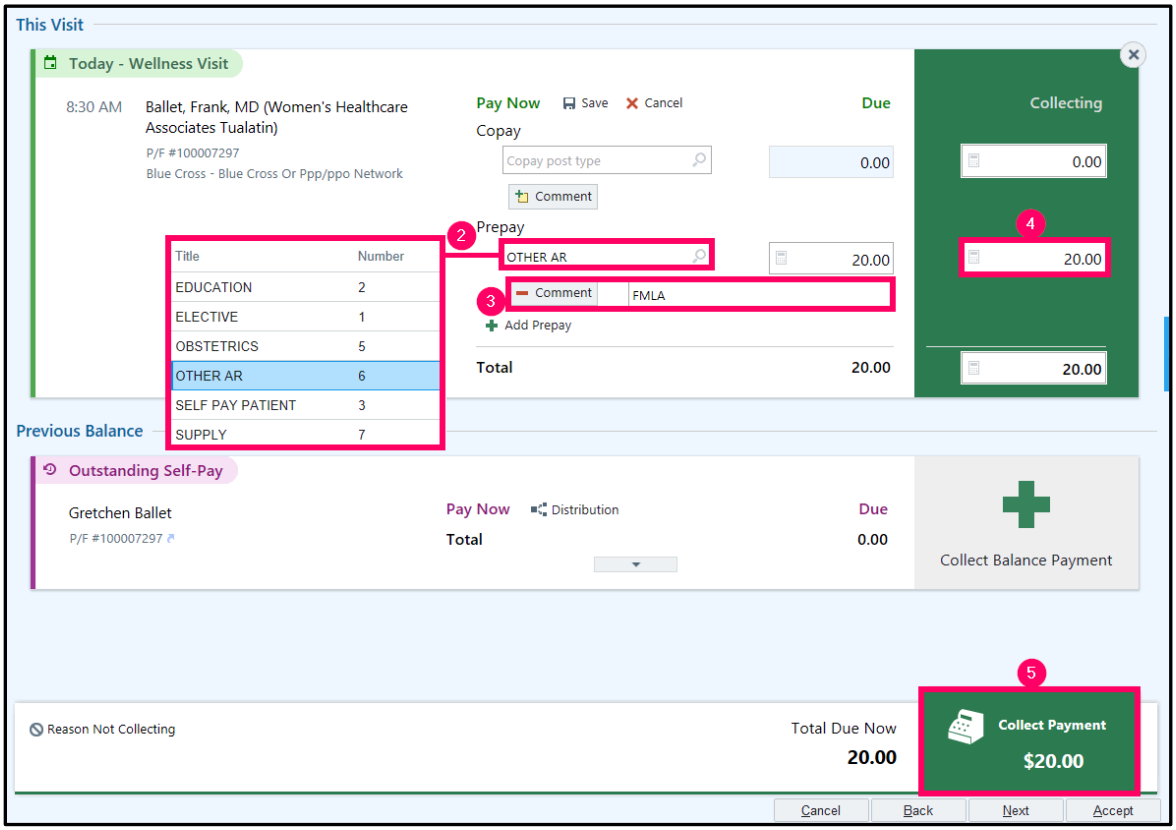
Prepay Payment Categories

****Staff Message the Billing and Coding pool if you add any addional payments not already displayed on the account (ex: FMLA)****

Payments Outside of Check In – Payment Collection	Payments During Check In
	
EDUCATION PREPAY – Childbirth Education Fee **ELECTIVE PREPAY** – Circumcision Deposit **OBSTETRICS PREPAY** – OB Self-Pay Deposit **PB FIRST** – Any copay or payment on account for services rendered **PREPAY OTHER** – CD's, FMLA/STD, Medical Records, US Flash drive, or Nitrous Fee **SUPPLY PREPAY** – Compression Socks **SURGICAL PREPAY** – Surgery Deposit	**EDUCATION** – Childbirth Education Fee **ELECTIVE** – Circumcision Deposit **OBSTETRICS** – OB Self-Pay Deposit **OTHER AR** – CD's, Medical Records, FMLA/STD, US Flash Drive, or Nitrous Fee **SELF PAY PATIENT** – GYN Self-Pay Deposit, BH Prepay **SUPPLY** – Compression Socks, Pessaries

Payments – *Adding a Prepay*





Prepayments will populate if there is an estimate attached to the visit (you can see details listed in the estimate). They can also be added manually. Prepayments include: Self Pay Deposits, Behaviorist Copays, Circumcision Deposits, Surgery Deposits, payment for Childbirth Education Class, Compression Socks

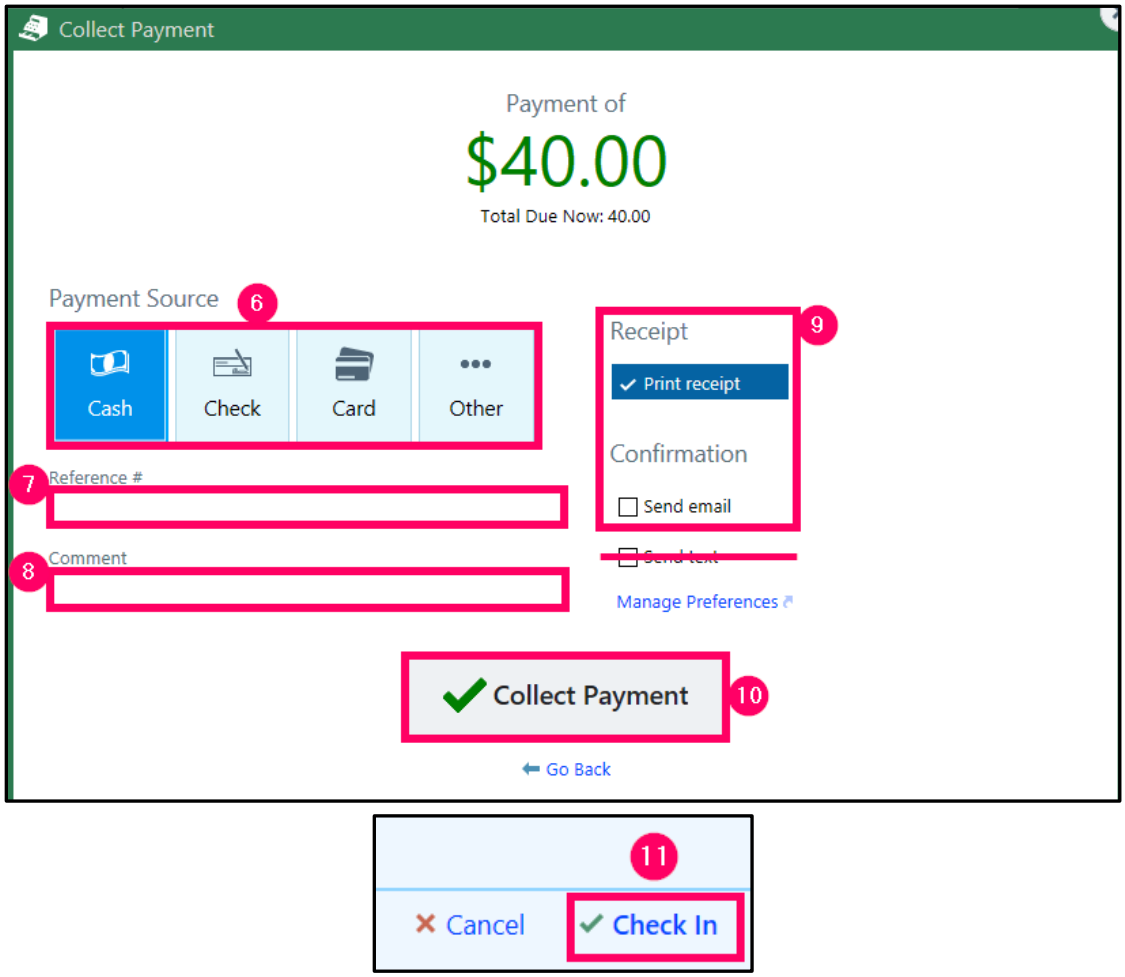
1. Click on **Add Prepay**

2. Select the **Payment Category** *(See page 25)*

3. Add a Comment regarding what the payment is for. *Example: FMLA*

4. Enter the **amount** the patient tenders

5. Click **Collect Payment**



6. Click on the form of payment that the patient pays with

7. **Reference** - If the patient pays by:
   *Check – enter the check number
   *Credit Card – enter the last four digits of the card number

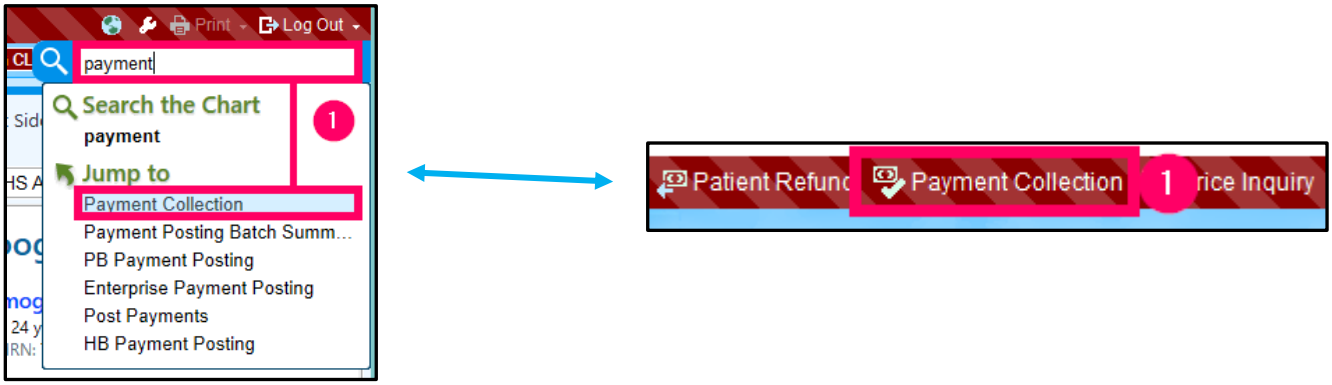
8. Add any **Comments** if needed

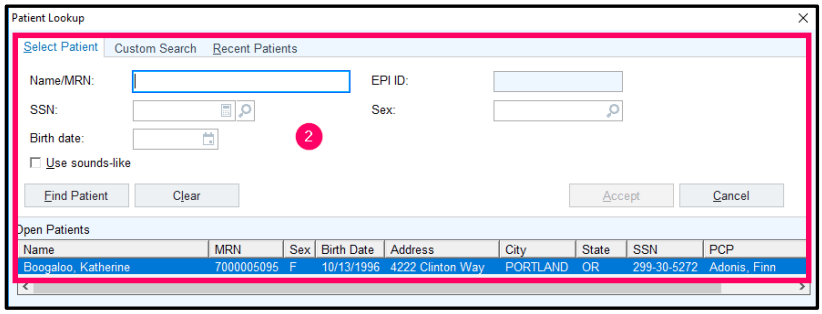
9. Always print the patient a receipt when taking a payment. You also send a copy of the receipt to the patient's email

10. Click **Collect Payment.** *Note: Card will prompt the credit card machine to function after selecting Collect Payment*

11. Click **Check In**. The receipt will print automatically

Payment Collection – *Collecting a Payment Outside of Check In*



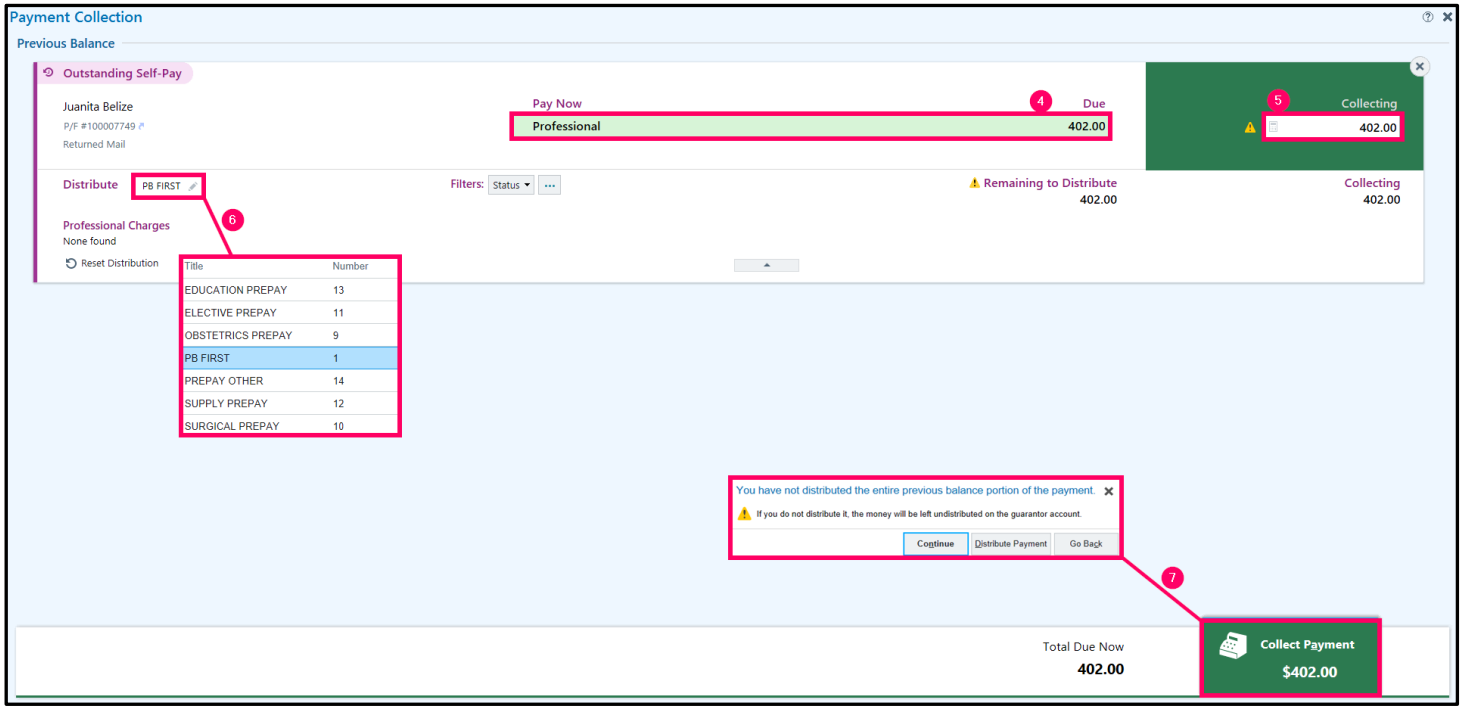




1. From the Search bar or from the Hyperspace toolbar, click on **Payment Collection**

2. Find the patient you are taking a payment for

3. Click **Collect Balance Payment**

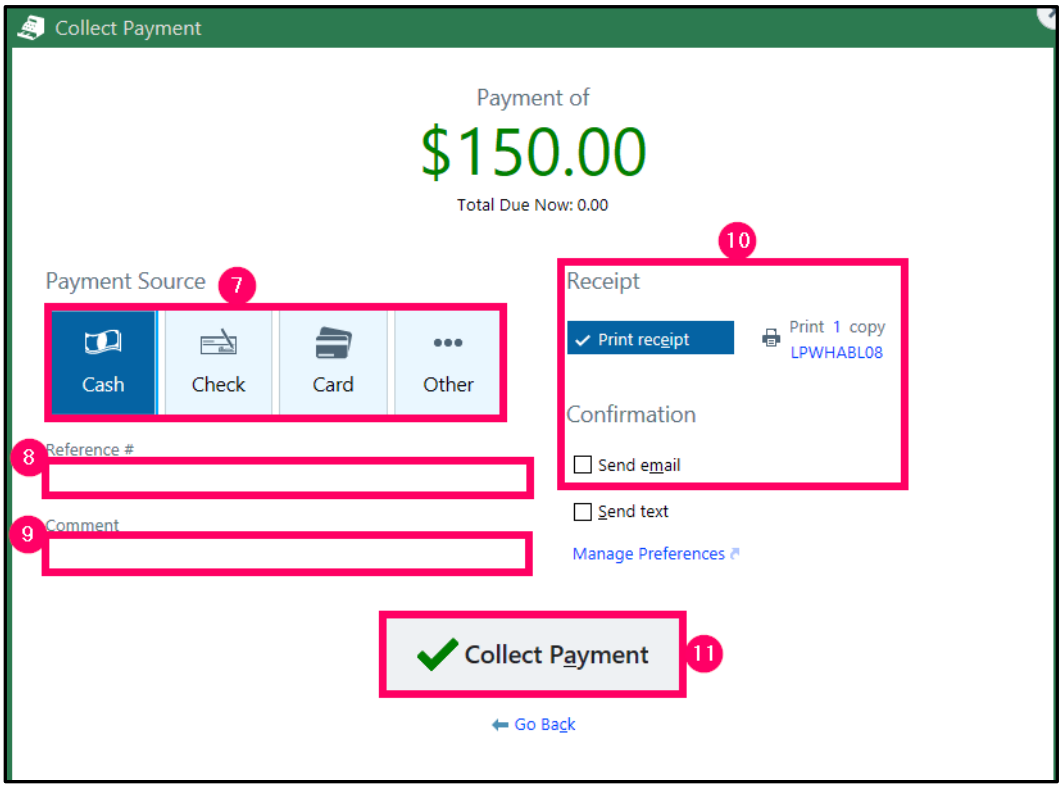


4. The patients current **balance due** will be listed here

5. Enter in the dollar amount the patient will be paying under **Collecting**

6. If you are collecting a payment that is not part of the current balance, change the **Payment Category** here. *Example: FMLA forms are listed under **PREPAY OTHER**. **See Page 26 for more Payment Category details*

7. Select **Collect Payment**. Continue through the Distribute Payment window. Payments will be distributed by our Financial Services Team on their end



7. Choose which form of payment the patient would like to use

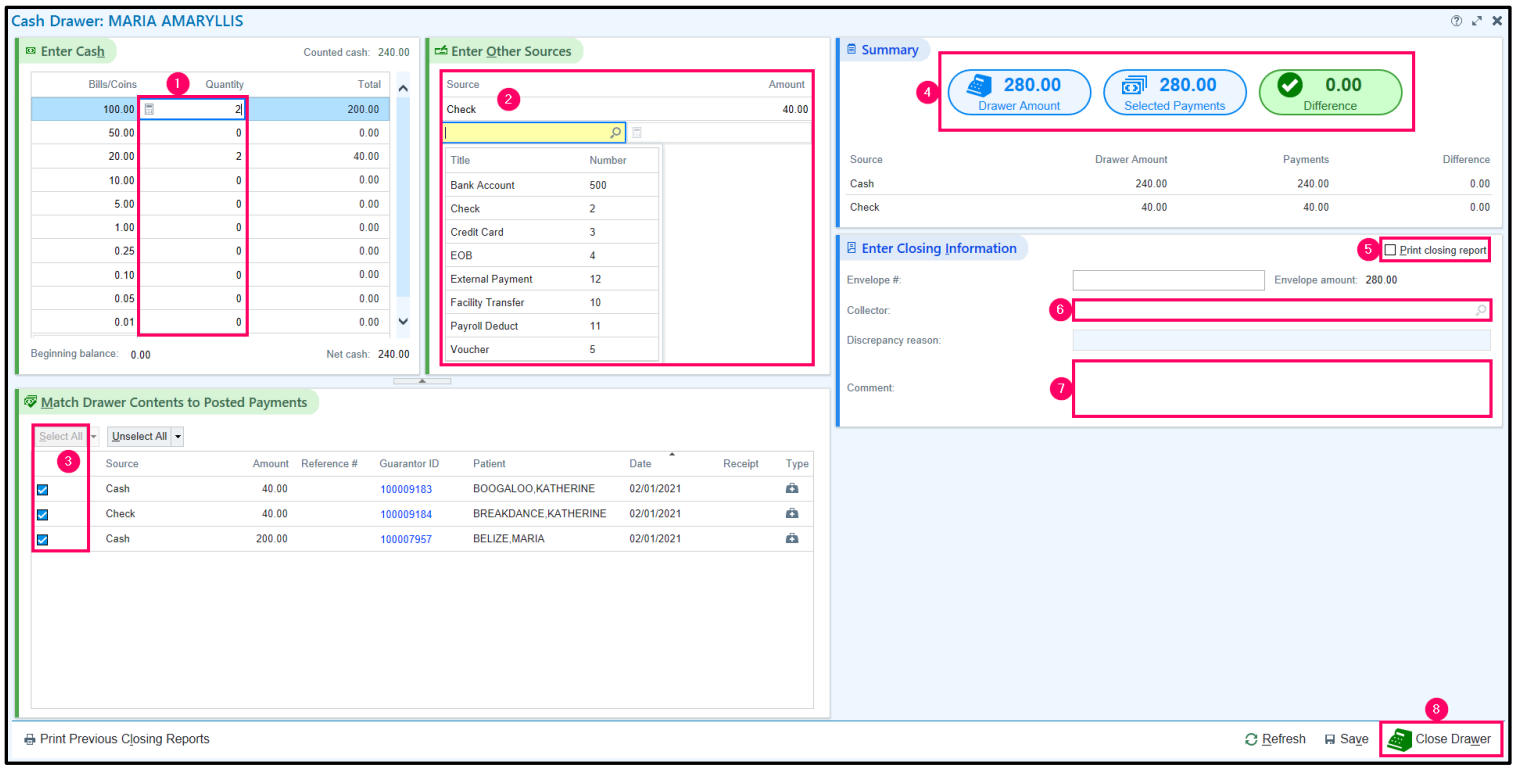
8. **Reference** - If the patient pays by:
   *Check – Enter the check number here
   *Credit Card – Enter the last four digits of the card number here 9

9. Enter a **Comment** if needed

10. Ask if the patient would like a Printed copy or an E-mailed copy of their receipt

11. Click **Collect Payment**

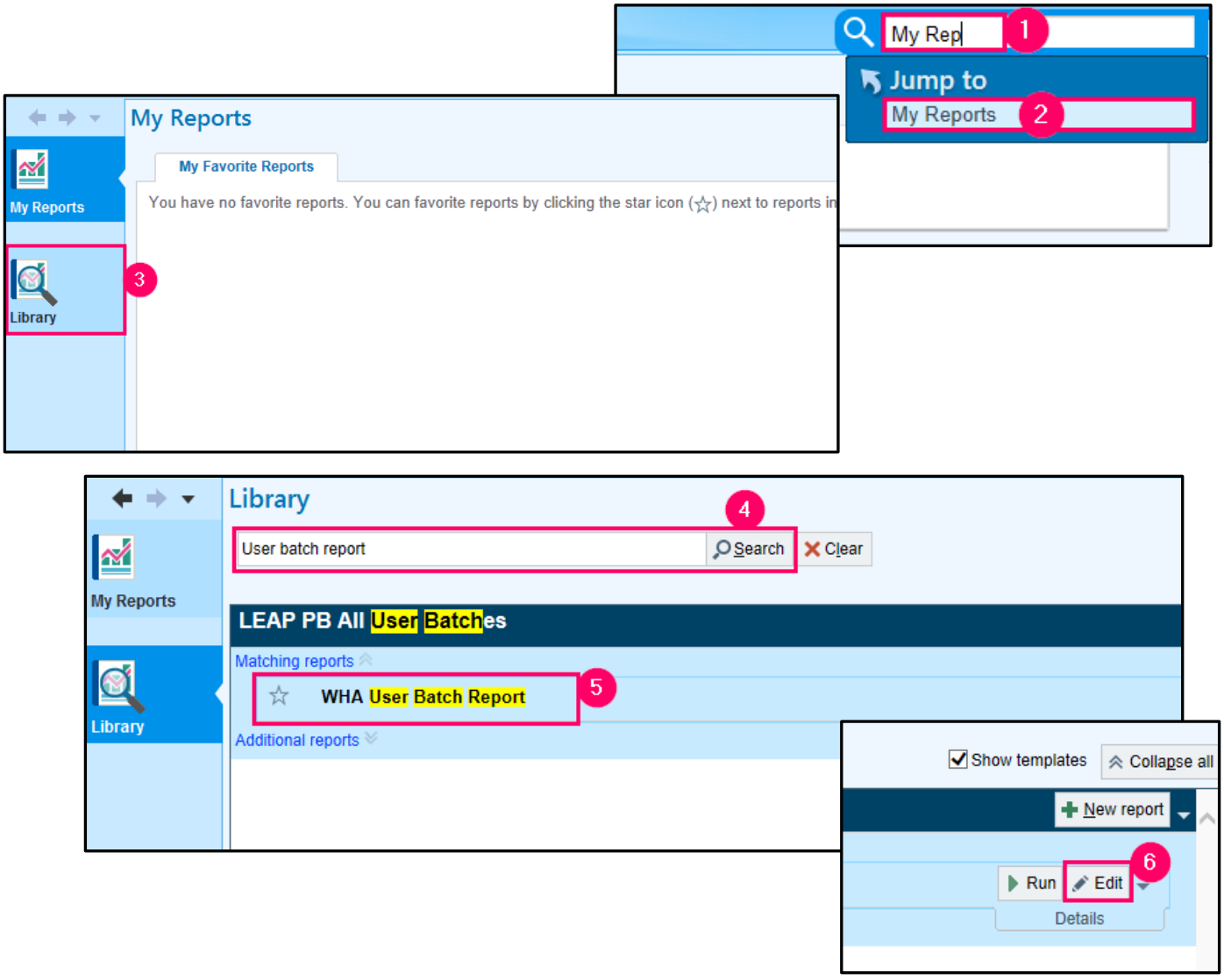
Cash Drawer



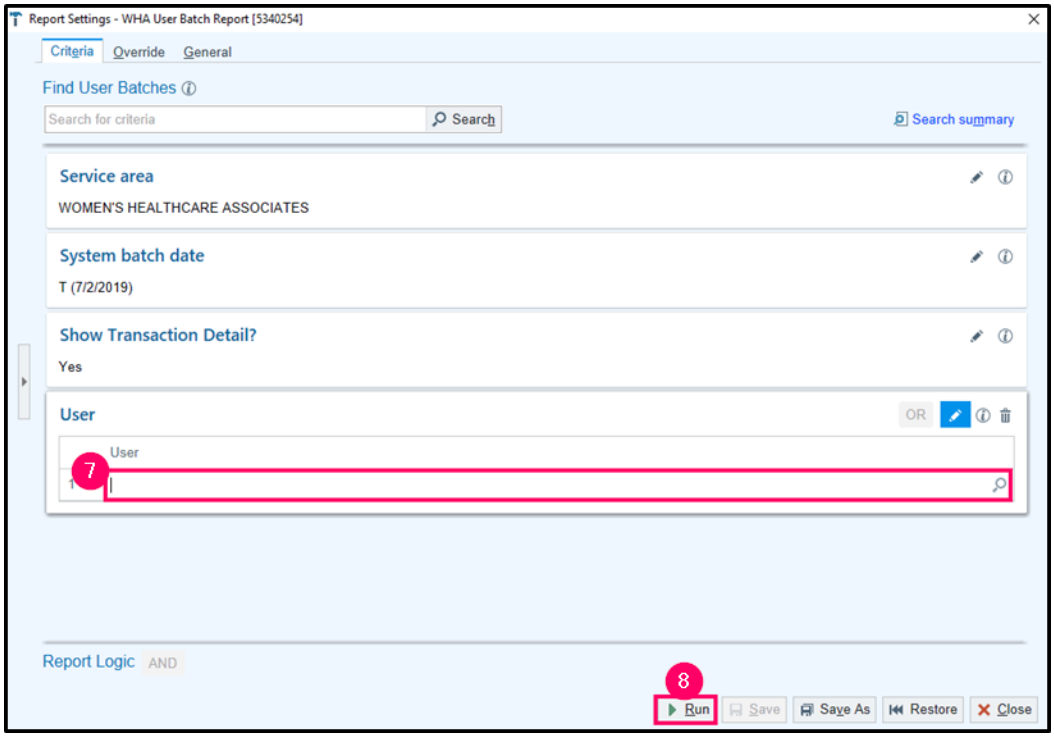
1. If you received cash, enter the number of bills or coins you have for each denomination
2. If you received check payments, indicate type of payment under **Source** and the amount received under **Amount**
3. All payments received will fall under Drawer Contents. Click **Select All**
4. The **Difference** should equal zero. If it does not, proceed to Step #5. If it does equal zero, proceed to Step #6
5. Checkmark the **Print closing report** box to ensure your batch prints when closed
6. Under **Collector**, enter the user who collected the payments (yourself)
7. Under **Comment**, enter the clinic and date (ex: TN 10/20/2023)
8. Select **Close Drawer** once finished
9. Click **Close Drawer**

Note: Only print and send your Cash Drawer if you received cash or check payments

User Batch Report



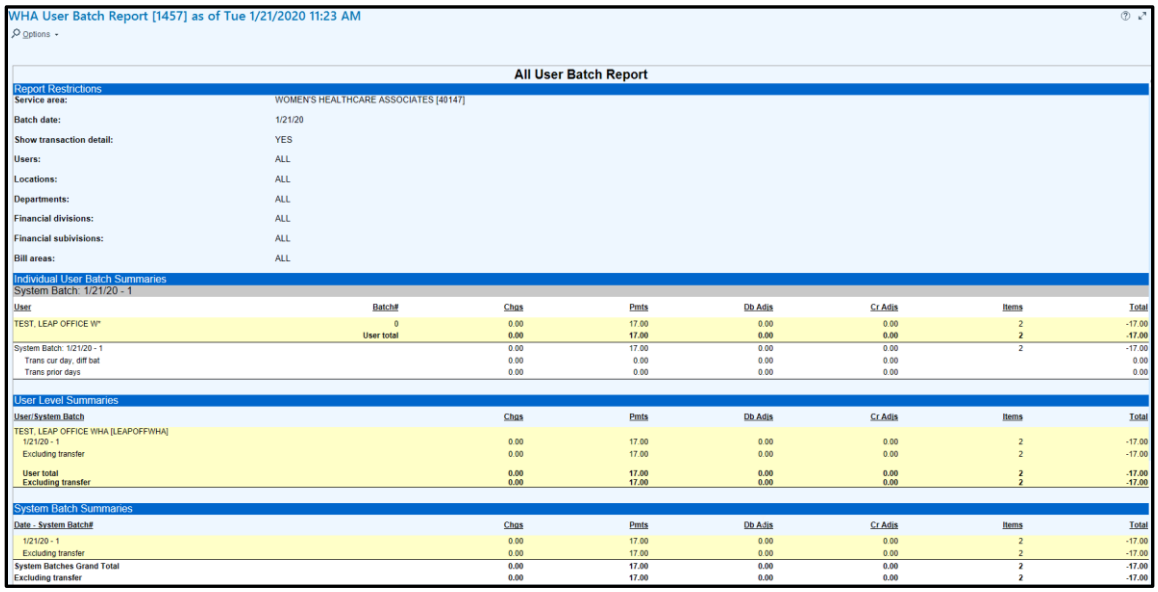
1. In the search field (top right-hand corner) begin typing **My Reports**
2. Click on **My Reports**
3. Click on **Library** on the left hand side
4. In the search field (under Library) type **User Batch Report**
5. Click on **WHA User Batch Report**
6. Click **Edit**

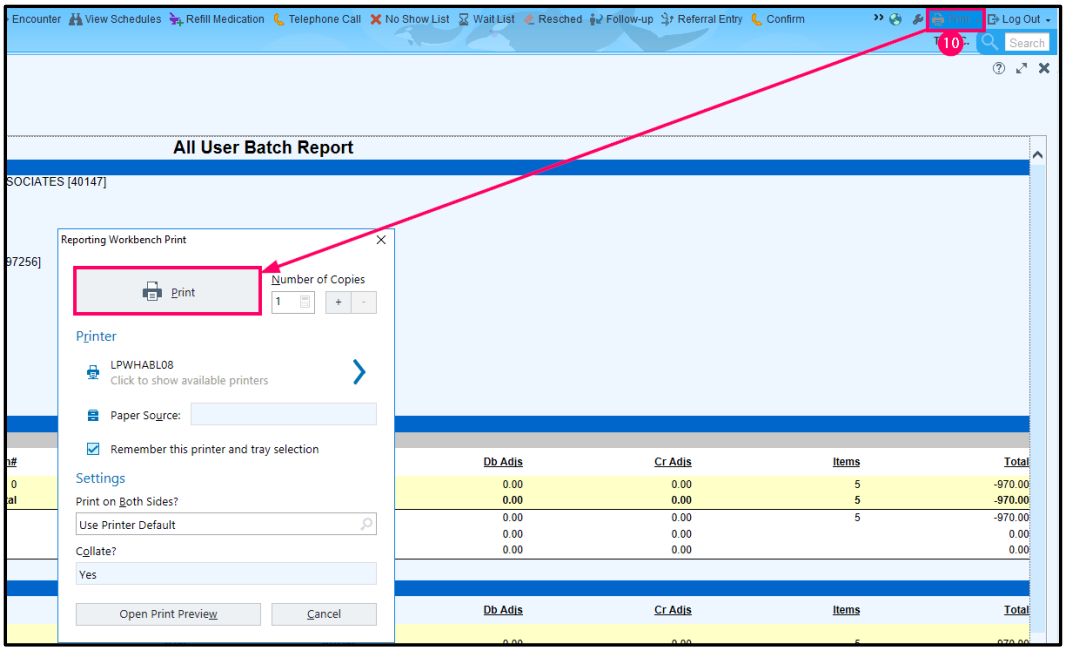


7. Enter your name into the User search field

8. Click **Run**

Your User Batch Report will pull up a summary. Click **Print** in the top right hand corner of the page



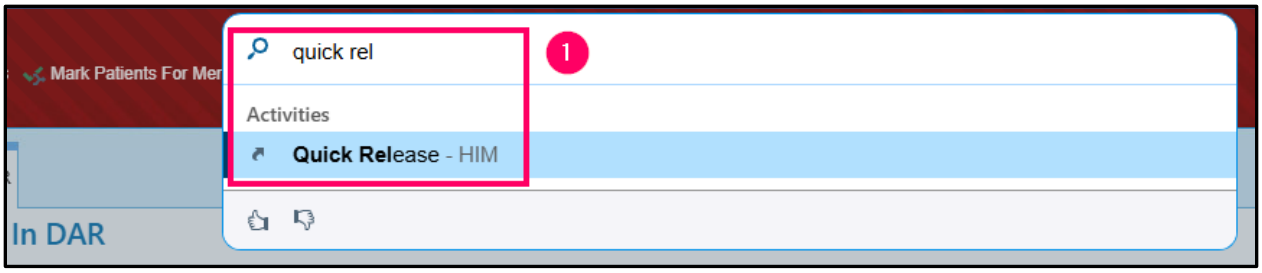


Your User Batch Report will pull up with a summary

10. Click **Print** in the top right-hand corner of the page. Print from the Reporting Workbench window and send as you would your Cash Drawer

Quick Release

Quick Release will only need to be utilized if you did not already document in a Telephone Encounter or if a Referral was sent out

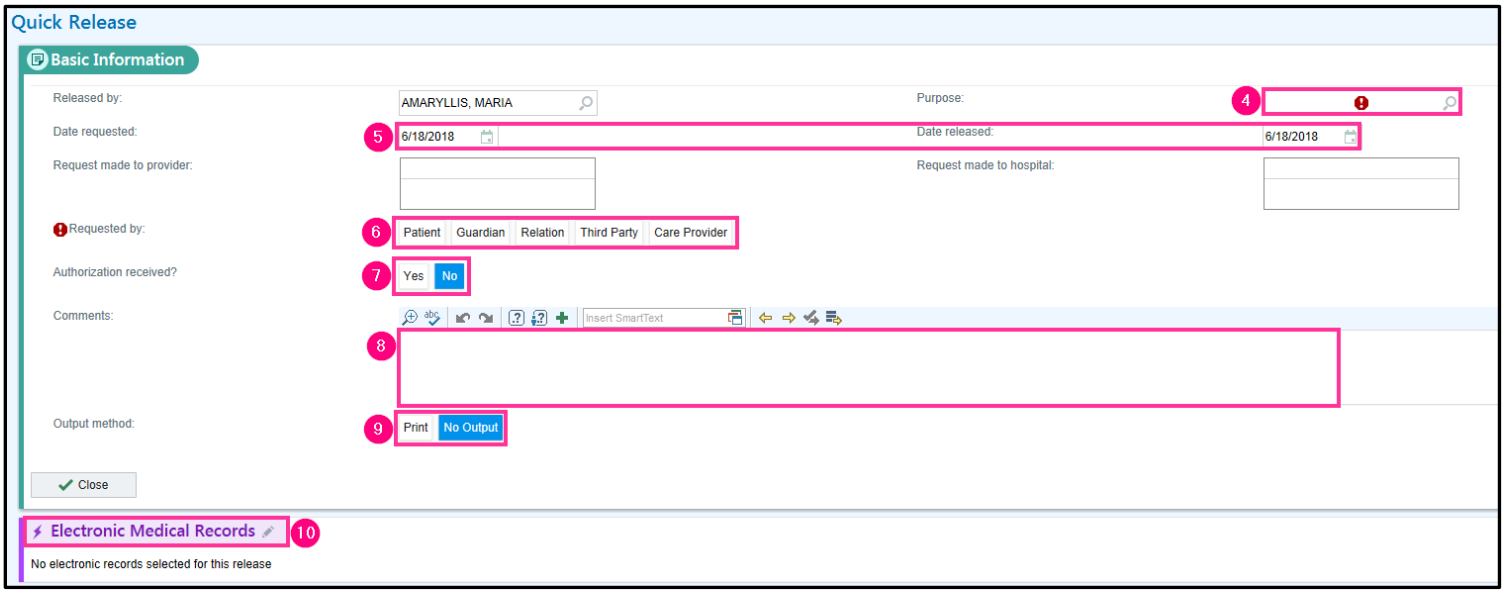




1. Search and select **Quick Release** in the top search bar

   *You can also find it in the Epic drop down button, under HIM
   *You can add this Quick Release button to your Hyperspace toolbar, too

2. Enter the patient's information to pull up a Quick Release for that patient



4. Enter the **Purpose** (reason for creating a Quick Release)
   *The most common category you will likely choose is: Continuing Medical Care

Note: For FMLA or Short Term Disability releases, you will only need to document in the Telephone Encounter so a Quick release is not needed in these cases

5. Adjust the **Date Requested** and **Date Released** if different than the default date of today

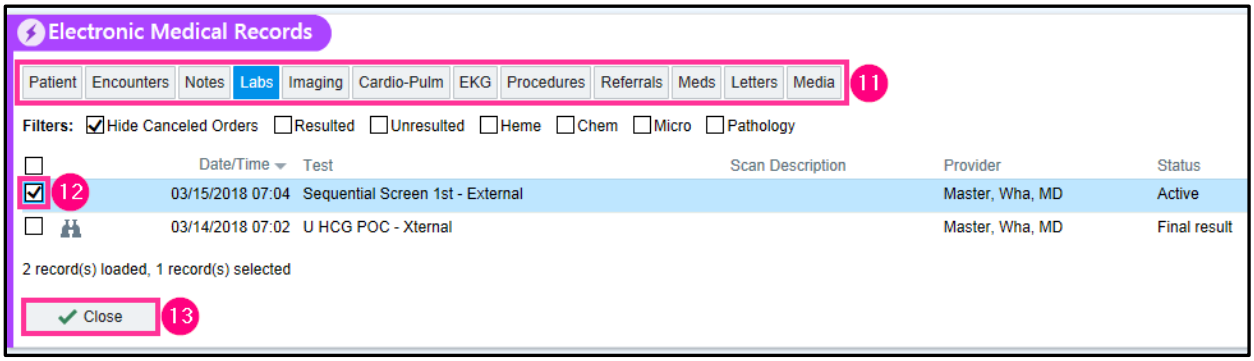
6. Indicate who requested the release of information.
   *If you pick "Patient", add any needed notes specifying who the information will be released to (see step #7)
   *If you pick "Third Party", enter the company in the **Name** field (the hard stop for the "Requester" field will disappear)

7. Indicate if you received a Release of Information from the patient

8. Add notes specifying any needed details of the release (ex: name of insurance company)

9. If you need to print records to send out, click **Print**. If you do not, (for example, you sent records the day before), click **No Output** and indicate in the "Comments" field what you printed/sent previously

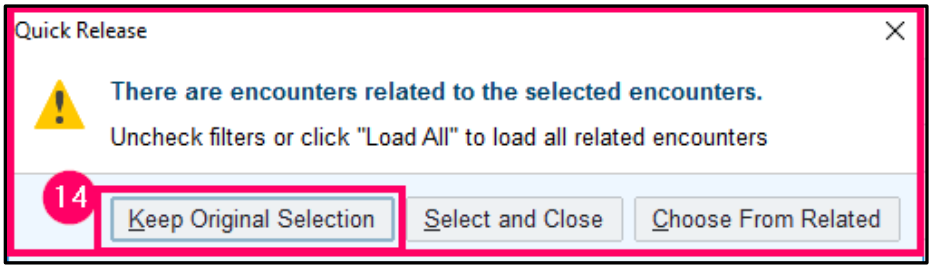
10. Click on **Electronic Medical Records**



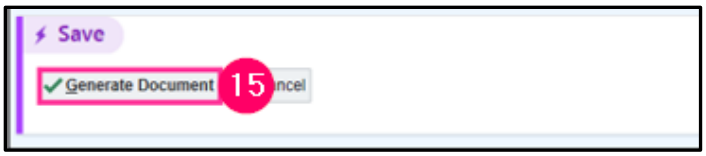
11. Click on the type of record(s) you need to release

12. Checkmark the record(s) you need to print (or printed on at a previous time)

13. Click **close**



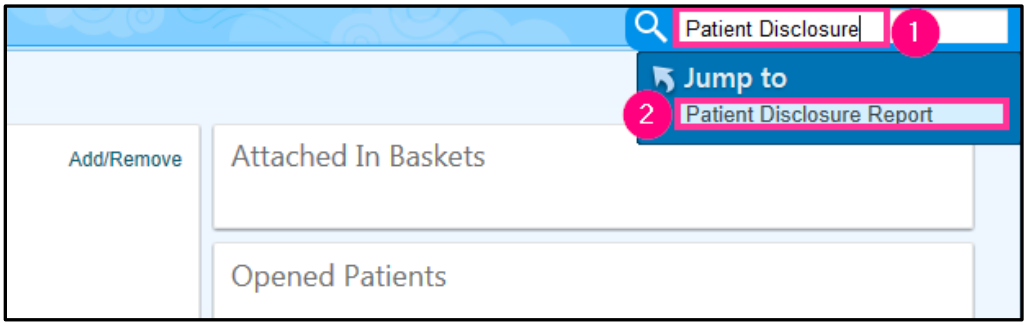
14. When this Quick Release window appears, click “Keep Original Selection”

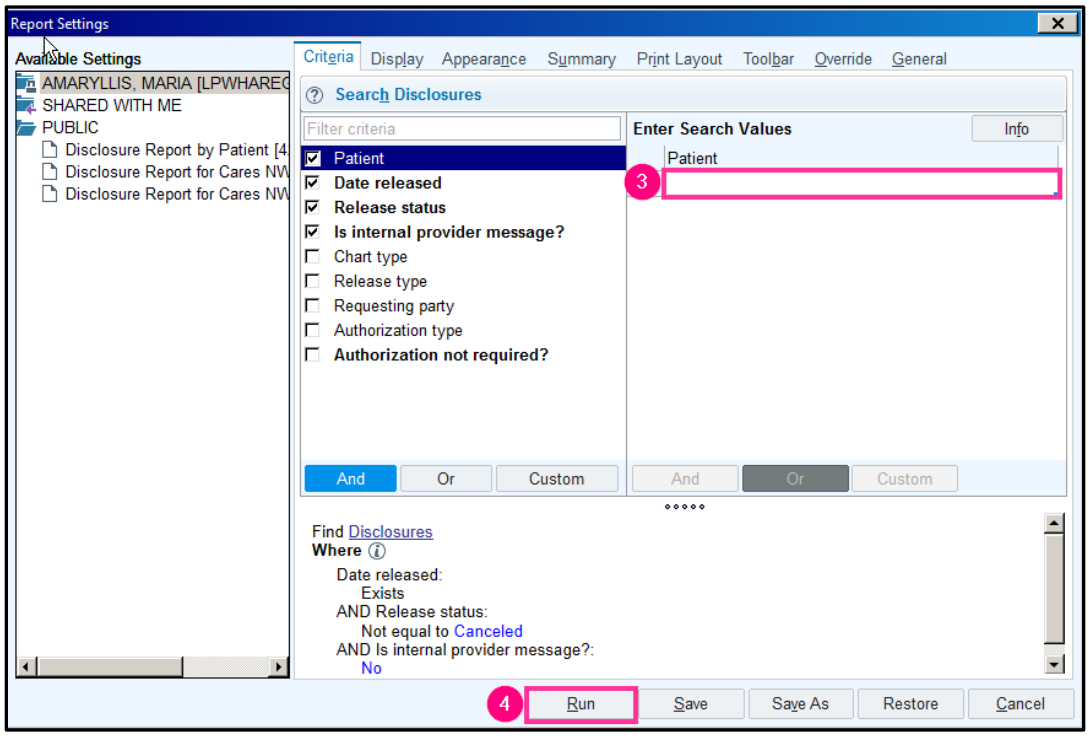


15. Click **Generate Document**

Patient Disclosure Report

To view any releases created or ROI inputted by HIM or other LEAP clinics



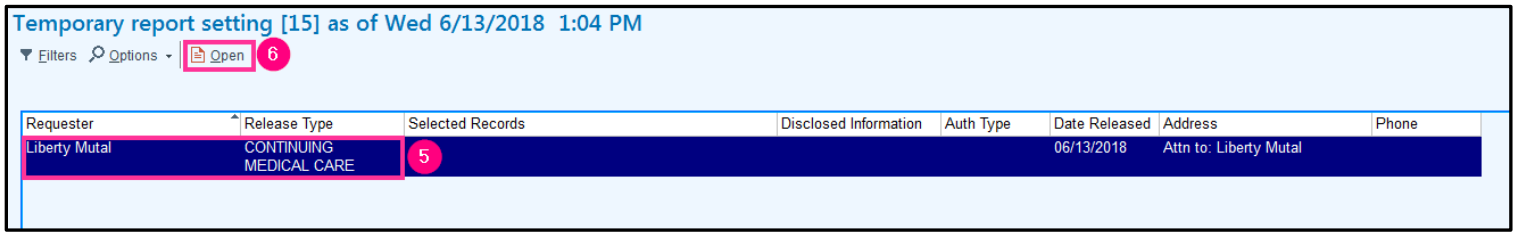


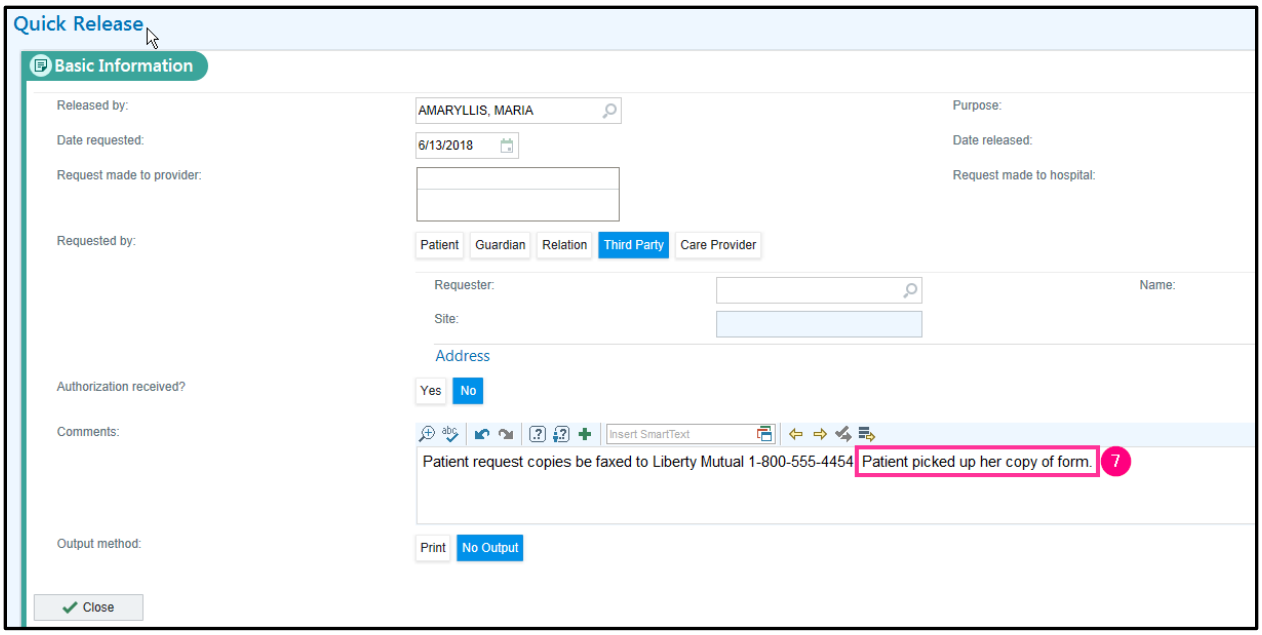
1. In the search field (top right-hand corner of the screen) start typing in **Patient Disclosure**

2. Click on **Patient Disclosure Report**

3. Enter the patient's name

4. Click **Run**







5. If there is more than one release, click to highlight the release that you want to see

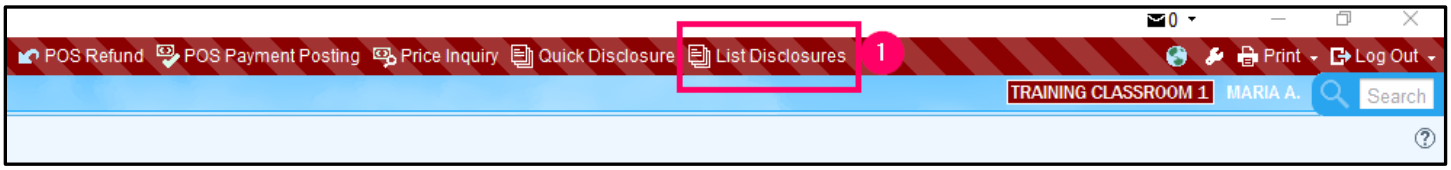
6. Click **Open** (you can also double click on the line highlighted in step #5 to open the
document)

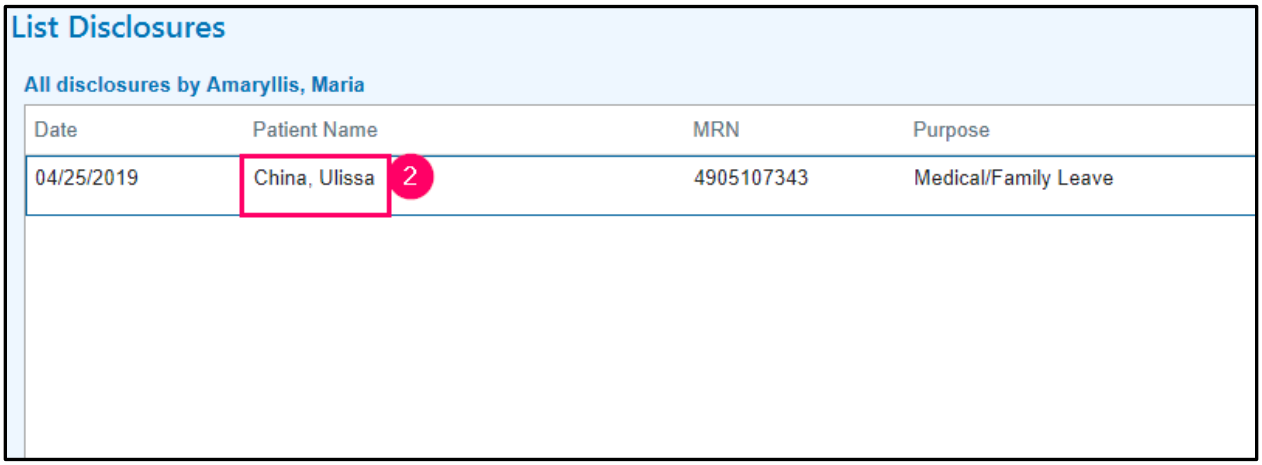
7. Update the "Comments" field as needed

8. Click **Generate Document** to save any changes

List Disclosures

Used for a Quick Release you created that day





1. Click on **List Disclosure** on your Hyperspace Toolbar (if it is not on your Hyperspace toolbar, you can search for it as well

2. To view a Quick Release in detail, click on the patient's name in blue font.
   *You can update the Quick Release if needed, such as if a patient picked up their paperwork or if forms were faxed, etc.